Therapeutic Autoethnography

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Therapeutic Autoethnography

"Be loyal to the story," […] "be eternally and unswervingly loyal to the story…"

Isak Dinesen in Hannah Arendt (1955, p.97)

Therapeutic Autoethnography

"When the storyteller is loyal… to the story […], silence will speak. Where the story has been betrayed, silence is but emptiness. But we, the faithful, when we have spoken our last word, will hear the voice of silence."

Isak Dinesen in Hannah Arendt (1955, p.97)

This paper outlines a qualitative research method, Therapeutic Autoethnography, which is an adaptation of Autoethnography, a qualitative research method currently applied across disciplines related to sociology, education, medicine, anthropology, and the arts. Therapeutic Autoethnography has defined parameters specific to studying psychotherapy processes in natural set-and-settings. Observations into the therapeutic process and results of the inquiry are derived from the phenomenological perspective of the psychotherapist, the practitioner of the studied therapy situation. Studies using Therapeutic Autoethnography are means to inquire into the process in the psychotherapy sessions. Results from the method help researchers understand long-standing questions, like therapist's effects and psychopathology, while enhancing theoretical understandings behind already established therapeutic modalities. Introduced here is a methodology defined for psychotherapy process research in naturalistic psychotherapy set-and-settings. The research method is potentially non-invasive and non-intrusive to the therapy subjects.

Therapeutic Autoethnography uniquely engages the therapist-practitioner as the researcher. This method is primarily designed for practitioners interested in being part of the research universe. The method has the potential of being accessible to practicing therapists who may not otherwise be inclined to take part in traditional forms of psychotherapy research while having invaluable insights from their experiences in practice to share. Therapeutic Autoethnography can be used as a standalone method or incorporated into the more extensive process and outcome studies, single case time series and case studies traditionally carried out by institutions. Illustrate this method. Two complete case studies (which can be read separately from the rest of the paper) and three recently published articles incorporating Therapeutic Autoethnography are featured here.

The role of psychotherapy holds more dominance and relevance than ever in post-modern westernized culture. Psychotherapy has long been established as an efficacious and cost-effective treatment for psychological and behavioral

Table 1 Psychotherapy outcome research generations (Lutz et. al 2021)

	Outcome Research Generations					
Bergin & Garfield Handbook Editions	**1st**	**2nd**	**3rd**	**4th**	**5th**	**6th**
Time frame	**1950s-1970s +**	**1960s-1980s +**	**1970s-1990s +**	**1980s-2000s +**	**1990s-2010s +**	**2010s-2020s +**
Theme	Justification	Specificity	Efficacy & cost effectiveness	Effectiveness & clinical significance	Evidence-based practice	Practice-based evidence
Thematic question	Is psychotherapy effective?	Which psychotherapy is more effective?	How can treatments be made more (cost) effective?	How can research findings be transferred into clinical practice?	How can the quality of treatment delivery (by clinical guidelines) be improved?	How can treatment outcome be improved?
Methodologies	Control group comparisons; effect sizes, meta analyses	Randomized control group trial; factorial design, placebo group.	Probit analysis; growth curves; structural modelling; power analyses.	Clinical significance meta-analyses	Meta-analyses; predictive modelling; patient-focussed research	Patient-focussed research; meta-analyses; predictive modelling; large (multisite) trials.
Key issues	Efficacy; spontaneous remission	What treatment. By whom, is most effective for this individual with that specific problem and under which set of circumstances?	Dose-response; medical offset; health economics; evidence-based medicine; low-cost interventions	User perspectives; evidence based practice; practice guidelines; treatment gap	Evidence-based practice; empirically validated treatments; routine outcome monitoring; good enough level of change	Patterns of change; routine outcomes monitoring; patient feedback; practice oriented research; low-cost interventions; therapist effects

dysfunction (Cuijpers et al., 2013; Lambert, 2013; Lutz et al., 2021). Psychotherapy is a socially negotiated and institutionally legitimated cultural object grounded on the values placed on it by the society that partakes in the activity (Gelo et al., 2016). Perspectives of practice and research in psychotherapy will remain diverse, despite the push for many in mental health practice and research to create a "generalized thesis" on what psychotherapy is. When the reality does not fit this generalized thesis, psychotherapy and psychotherapy practitioners bear the brunt of the controversy (Garfield, 1983; Luborsky, 1954). This plurality extends to the varied intentions of psychotherapy researchers. The individual researcher's attitude to the object of their study is— whether one would explicitly admit to it— a subjective response to their worldview. Research and practice in psychotherapy can be an uncolonized, dynamic, contemporary exchange of self-reflection and experiences of the field when the writings are also self-reflective and open to diverse subjective expressions on the subject by its practitioners.

Colonization is the act of appropriating a place or domain for own use without understanding or integrating oneself with the field of psychotherapy when the dominant opinion of a powerful vocal few in the research field decides which empirical evidence in psychotherapy research is to be valued or rejected. Psychotherapy researchers, moving forward, need theory as a resource (Salvatore, 2011). When theory is hypothesized, it will help researchers to relate this theory to the contemporary reality of psychotherapy practice. Contemporary psychotherapy training and practice have inherited pearls of aesthetic wisdom from mentors from a lineage long before Freud (Ellenberger, 2008). Throughout their professional careers, psychotherapists upskill through the flow of learning and sharing from mentors and colleagues. Intentionally relating to the "what is" in the naturalistic setting in the psychotherapy session keeps the knowledge base of practice current. Research in psychotherapy, if it were to be useful to the modern-day practitioner, needs to allow for this kind of flow. While the current attitude of research is moving towards this kind of attitude that aligns with the reality of the field, sectors of the research community still seem colonialist. We may be able to mediate between the restrictive position of research and the needs of practice with the introduction of new research methodologies like Therapeutic Autoethnography.

Generations of psychotherapy research efforts have helped us realize how complex psychotherapy is. The seventh and latest edition of *Bergin and Garfield's handbook of psychotherapy and behavior change* (Lutz et al., 2021) impresses upon us how psychotherapy research has evolved through their editions since 1971. Generations of psychotherapy research have taken us beyond the need for justification of psychotherapy of the 1970s to efficacy and evidence-based practice of the late 20th Century to a move towards practice-informed evidence of the 21st Century—the methodology shifts from purely quantitative to including important qualitative modes of inquiry (see table 1 & 2). Current trends in psychotherapy research have advanced to the focus on drawing evidence from routine practices that reflect how

Table 2 Psychotherapy process research generations (Lutz et. al 2021)

Process Research Generations						
Bergin & Garfield Handbook Editions	**1st**	**2nd**	**3rd**	**4th**	**5th**	**6th**
Time frame	**1950s-1970s +**	**1960s-1980s +**	**1970s-1990s +**	**1980s-2000s +**	**1990s-2010s +**	**2010s-2020s +**
Thematic question	Are there objective methods for evaluating process?	What components are related to outcome?	How does change occur (via a qualitative approach)?	How does change occur (via a qualitative approach)?	How to disentangle common and specific factors of change?	How to identify moderators, mediator and mechanisms of change?
Methodologies	Random sampling of sections of therapy; 'uniformity myth' assumed; therapist patient post-session evaluations	Single-case methodologies; sampling 5-minute sections of therapy	Taxonomies; linking process to outcomes	Qualitative methods; narrative approach; discourse analysis; descriptive studies; theory development	Additive and dismantling trials; factorial designs; shared components	Moderator, mediator analysis; Between and within patient process outcome relations
Key issues	Verbal & speech behaviours	Rogerian facilitative conditions (e.g. empathy)	Therapeutic alliance; verbal response modes	Events paradigm; single case approach; qualitative methods	Common factors; specific factors	Moderators; mediators; sudden gains; mechanisms of change

therapies are delivered in everyday situations, which involve studying change in diverse and naturalistic situations (ibid. p. 11).

The psychotherapy process is multidimensional, multi-determinate, multi-faceted, and imbued with reciprocating interconnected influences constantly in flux (Hayes et al., 2007). Change in psychotherapy, being non-linear, discontinuous, and cognitively unpredictable, is the sought-after outcome of psychotherapy. Change is also a concept that remains ungraspable to the psychotherapy researcher (Salvatore et al., 2015), who needs to gain first-hand experience in practice. Researchers have accumulated a plethora of data that still need to adequately comprehend the phenomena of the psychotherapeutic relationship (Beutler & Harwood, 2002; Orlinsky et al., 2004).

Appreciating psychotherapy's complex and dynamic character and psychotherapeutic change is a step forward in setting a new paradigm to research psychotherapy (Gelo & Salvatore, 2016). Meta-theory informs us of the phenomenon's complexity but does not provide an understanding of that phenomenon. What is complex becomes ultimately reduced when rendered in research by postpositivist thinking that has been the "invention of science" adopted since the 17th Century (Wootton, 2015). Psychiatry and many clinical psychology practices attached to the "evidence-based" paradigm seem to be products of this postpositivist paradigm, where the fullness of experience gets hacked into discrete, quantifiable parts, which in research are numbers. The organism of psychotherapy— and the psychotherapists and clients— has shown to not function in this realist-postpositivist realm.

Psychotherapy is a continuation of a long tradition of human wisdom where truths are shared and understood holistically through the wholeness of experiencing life which cannot be the same when split into the sum of its parts (Ash, 1998). The transfer of experiential knowledge in psychotherapy has traditionally happened. The flow of aesthetic knowledge happens through telling stories, myths and metaphors, and this is the ontological and epistemological paradigm of psychotherapy and its practitioners in practice. Current research is still disconnected from the overarching psychotherapy research paradigm. There are, however, signs of correction in recent years from this situation, with the increase in qualitative research (Gelo et al., 2019), which is directed away from postpositivism, moving towards producing knowledge from human experience. The call is then for modern-day practitioners and researchers to embrace both paradigmatic form (seeking to explain abstract laws) and narrative ways (through contextualized concrete stories) of knowing (Madill & Gough, 2008). A turn away from postpositivist predominance in research, a movement towards methodological pluralism (Slife & Gantt, 1999), and a qualitative research methodology injected with an attunement to aesthetics, may resolve the practice-research dissonance (Levitt, McLeod, & Stiles, 2021).

In the current universe of common qualitative research methods adopted by psychotherapy research (which is presented in table 2), Therapeutic Autoethnography can be placed as a case study method based on Autoethnography. Case studies are methods designed to develop an understanding of an episode, a therapy situation, or a person (in the case of Therapeutic Autoethnography, the client) or to disprove an extant theory.

Autoethnography as a research method has already been applied to psychotherapy. There are several notable works. Siddique (2011) applied the method to study a residential mental health project for women experiencing emotional or psychological crises by undertaking a role in the institution as a research project worker. The field of this work is the institution. Grant (2010) writes in prose and story about his battle with alcoholism over two decades. His story describes his experiences of feeling stigmatized and treated as 'other' by members of the humanistic counselling and therapy fraternity. The field in this work is a fraternity. Jones (2015) describes the therapeutic change process in somatic psychotherapy from the perspective of a client. The field, in this case, is the therapeutic encounter. Talbot (2018) writes about their experiences as a gestalt psychotherapist who became a full-time mediator, describing how they have tried to deepen the theoretical underpinning of mediation with a gestalt dialogic-relational approach and applying their skills and experience as a gestalt psychotherapist to being a mediator. The field of that study was the environment and professional relationships around the writer.

This paper's methodological approach to applying Autoethnography in psychotherapy research is differentiated from the above-mentioned published works. The differences are that in Therapeutic Autoethnography:

1. The method applies specifically to inquiry into the psychotherapy process.
2. The practitioner or clinician is the Autoethnographer-researcher.
3. The therapeutic encounter defines the field.

On preparing this dissertation thesis, there has been no article known to the author that describes a similar use and application of Autoethnography, apart from those published by the author.

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Table 3 Common Qualitative Methods (Levitt et. al, 2021)

Method	**Typical aims of the method**	**Perspectives of the researcher**	**Participant (Data sources) characteristics**
Phenomenology	To develop a description or an understanding of a lived experience.	The use of bracketing or epoché, the suspension of researcher's beliefs. Researcher's emphasize owning their own perspectives and their influence through the research process.	Participants need to be descriptive and reflective about internal experiences.
Grounded Theory	To develop a theory or an understanding of a research topic that is grounded in data.	Varied. The researchers are cautioned initially to avoid reading research literature to minimize bias, though this is disputed as doing so helps researchers sensitize them to the phenomenon. Approaches to Grounded Theory exist from across epistemological perspectives.	Typically the participants need to be descriptive about the research topic.
Narrative Methods	To examine the formulation, effects or structure of stories or identify the themes associated with the research topic.	Varied. Some theory-driven approaches. Approaches to narrative analysis exist from across epistemological perspectives.	Typically, participants need to convey the stories about the research topic for analysis.
Conversational Analysis	To examine the moment-by moment evolution of talk and its organization as it functions in relationships, social systems and institutions.	Varied. Approaches to conversational analysis exist from across epistemological perspectives.	Data are collected from across a specific type of conversation for analysis.

To be continued.

Table 2 (continued)

Method	Typical aims of the method	Perspectives of the researcher	Participant (Data sources) characteristics
Ethnographic & Autoethnographic Analysis	To identify cultural themes and process in everyday life experiences of a specified individual or group.	Typically approaches are from within constructivist and critical perspectives in psychology.	Data are typically collected by researchers living within a culture or a situation over a period of time. Diaries, cultural artefacts and field notes are often used.
Task Analysis	To identify the stops or stages via which tasks are performed successfully.	Research is grounded in a model of dialectical constructivism in which researchers begin with a theoretical rational model that is then transformed through empirical analysis.	Data comprise successful and unsuccessful tasks that can be subjected to analysis.
Case Studies	To examine a case to develop an understanding of a person or episode or to disproves an extant theory.	Approaches to case studies exist from across epistemological perspectives.	Data must provide detailed description of a case and its context.

Methodology

"Grown-ups like numbers. When you tell them about a new friend, they never enquire into what matters most. They never say to you: "How does his voice sound? What are his favorite games? Does he collect butterflies?" They ask you: "How old is he? How many brothers has he got? How much does he weigh? How much does his father earn?" Only then do they think they have an idea of him. If you say to grown-ups: "I've seen a beautiful house of pink brick, with geraniums at the windows and doves on the roof..." they can't picture this house. You have to tell them: "I saw a house worth a hundred thousand francs." At that point they exclaim: 'Isn't it pretty!'"

The Little Prince (de Saint-Exupéry, 1942/2015, p. 15)

Research Objectives and Aims

Therapeutic Autoethnography is a method that involves the recruitment of the practitioner psychotherapist as the researcher and observer in the field of the therapeutic situation with their client(s). Like an ethnographer, the therapist records their experience in that therapeutic setting with the client. This lived experience is in effect, the day-to-day activity of the practitioner in practice, and it is thus a process that happens in nature. We realize the main objective of this method when the practitioner can transfer, as fully as possible, their phenomenological experience of being that practitioner at the studied therapy sessions through their reflexive writing of the case study. A target audience that stands to gain from this research method is portions of the research community who are interested in real-life practice-based phenomena of psychotherapy, understanding the diversity of these phenomena, and studying therapists' effects qualitatively. The primary beneficiaries ultimately are practicing psychotherapists, whose training and continued upskilling happens through receiving and sharing stories from other therapists and mentors.

The Research Paradigm

Therapeutic Autoethnography is situated upon the ontological and epistemological perspective of constructivists-interpretivism, where knowledge is co-constructed and constantly (re)generated through ongoing dialogue between interested individuals. The process is heuristic (Moustakas, 1990). There is a relationship between the researcher and the field, which is the subject of study. Understanding through the process of research is the gist of the work. The logic of inquiry is abductive, and the truism is an inter-subjective affair. This postmodernist paradigm opens all modes of inquiry to critique, where all claims of truth become suspect of serving particular interests. As such, no method or theory can claim authority over knowledge. Heuristic methodologies allow the researcher to share their knowledge about something without claiming to know everything.

The research method of Therapeutic Autoethnography is qualitative as well as art based. Aesthetics is transmitted through the writing is creative and also analytical. Through this form of writing, the essence of lived experiences becomes sharable. The researcher that uses aesthetics can transfer information that is otherwise undefinable. Richardson & St. Pierre (2018) terms this CAP (creative analytical processes ethnographies). They call it a "new species" of qualitative writing that adapts to the socio-political climate in which we live, a world that is increasingly aware of the uncertainty behind "truths".

Therapeutic Autoethnography aims to realize the nature of psychotherapy in its naturalistic setting and what happens in daily practice. Psychotherapy is an open and complex dynamic system, where external factors affect what happens to the process. Most research methodologies interfere with the psychotherapeutic process that it

studies. This method is designed to be non-invasive to the psychotherapeutic encounter. In Therapeutic Autoethnography, the therapist, at the time of the session, often needs not know that the particular therapy situation will become the research subject.

Autoethnography

Autoethnography, as outlined in (Adams et al., 2015; Denzin, 2013; Ellis, 2004; Poulos, 2021), is the framework and philosophical foundation for the methodology of Therapeutic Autoethnography. Autoethnography is traditionally a method created for social and cultural studies. To adapt the methodology to psychotherapy process research, we assign Therapeutic Autoethnography for the method and provide clear definitions and placement of the researcher and the field. Using Autoethnography provides an already established scholarly reference for the design structure of Therapeutic Autoethnography without having to build up a foundation from scratch.

Autoethnography is a method that harnesses the power of the aesthetics of the atmosphere of the studied field, a means of mining and producing knowledge that transcends facts. Aesthetic evaluation is a pre-reflexive and pre-verbal process of sensing the atmospheric content of a situation. By using Therapeutic Autoethnography to study the psychotherapy process, doors open to the phenomenal field of the psychotherapeutic process, widening the scope of our understanding of psychopathology and the aesthetics of therapeutic contact.

What is Autoethnography?

The word autoethnography is composed of 3 parts which, in Latin, are *autos*, *ethnos* and *graphia*, meaning “the self”, “culture”, and “writing”, respectively. Autoethnography is a qualitative research method that investigates human processes, cultural and societal. Autoethnography is an observational, data-driven phenomenological method of studying a field. Integral to the process of doing autoethnography is its reflexive, personal creative writing, an aesthetic art form, story, or poetry. Autoethnography is an art-based method of research. Through this method, we illuminate social, emotional, theoretical, political, and cultural aspects of human life. Unique to autoethnography is the use of aesthetics to provide analyses and communicate results. The aesthetics are formed from writing poetic, impressionistic, and even lyrical stories, evoking feelings that reflect the tales of human existence. Autoethnography requires 1) the use of the researcher’s personal experience to describe phenomena, 2) the attention to and acknowledgement of differences between the researcher and those studied, 3) the reflexivity, deep and careful self-reflection on the part of the researcher to name and interrogate the differences at the boundary between self and others, 4) The revealing of the story of the life of others, including the 4) the balancing of intellectual and methodological rigor, emotion, and

creativity 5) the strife for the betterment of life (Adams et al., 2015; Bochner & Ellis, 2016; Chang, 2008).

The researcher takes a phenomenological approach to observing the field, being neither positivist nor postpositivist. Autoethnographic research aims to present possibilities of how situations and phenomena can arise. It does not aim to measure the probability of the phenomenon. Such research focuses on individual experiences, differences, and knowledge. Creating generalized assumptions about populations is avoided. In autoethnography, simply reporting results is not done. Observations and results from data collected are transmitted through evocative forms of writing. The results presented are neither statistical nor predictive but interpretative-suggestive. The researcher presents a self-reflexive and subjective insight into their observations. The story is not about the field but the observer-researcher who has entered the field. The results are grounded in the observation and participation of the observer-researcher, not in quantifiable data. The derivation of knowledge from the research is driven by memory, insights and felt sense rather than through generalizations.

Table 4 What Autoethnography is (Poulos, 2021, p. 23)

Autoethnography is	Autoethnography is not
Qualitative inquiry	Quantitative research
Phenomenological	Positivist or postpositivist
Focused on local knowledge	Focused on generalized knowledge
Evocative writing	Reporting
Interpretive or critical	Statistical
Suggestive	Predictive
Self-reflexive and subjective	Outer-directed or objective
Grounded in participation and observation	Grounded in quantifiable data
Driven by memory, story, experience	Driven by statistical generalizations or categories
Narrative, poetic, evocative writing	Research reporting
Seeking possibilities	Seeking probabilities
Written in first-person voice (generally)	Written in third-person voice

based on statistics. The end product of the research is not the report written in the third person voice but an evocative narrative art form written in the first-person voice.

Keleş (2022) quotes the authors' description of Autoethnography as a "qualitative research method that uses ethnographic methods to bring cultural interpretation to the autobiographical data of researchers with the intent of understanding self and its connections to others" Chang (2008, p. 56). As a "form of writing about the personal and its relationship to culture" Ellis (2004, p. 37), it is a cultural analysis that is transmitted through personal narrative (Boylorn & Orbe, 2014, p. 16), it is "part of a corrective movement against colonizing ethnographic practices that erased the subjectivity of the researcher while granting him or her absolute authority for representing 'the other' of the research." Gannon (2006, p. 475).

Autoethnographic writing is a specific genre of academic writing and research that opens and "displays multiple layers of consciousness, connecting the personal to the cultural" (Ellis & Bochner, 2006, p. 739). It is "a form of self-narrative that places the self within a social context. It is both a method and a text" (Reed-Danahay, 1997, p. 6), "a self-narrative that critiques the situatedness of self and others in social context" (Spry, 2001, p. 701). It is "an intriguing and promising qualitative method that offers a way of giving voice to personal experience to extend sociological understanding" (Wall, 2008, p. 38), "a process of self-exploration and interrogation [that] aids individuals in locating themselves within their history and culture [,] allowing them to broaden their understanding of their values in relation to others" (Starr, 2010, pp. 1). Holman Jones (2005) calls Autoethnography, "a blurred genre . . . a response to the call . . . it is setting a scene, telling a story, weaving intricate connections between life and art . . . making a text present . . . refusing categorization . . . believing that words matter and writing toward the moment when the point of creating autoethnographic texts is to change the world" (Holman Jones, 2005, pp. 765). Exploring the writings published in the online autoethnography magazine, The Autoethnographer [1], one may come to realize how varied the application of autoethnography is.

Writing as Method of Inquiry

The writing process is an integral aspect of the research process in Autoethnography. It serves as the mode of inquiry that happens throughout the research process. Autoethnographic writing is a genre of academic writing that is autobiographical. The data from the field drives the inquiry and is transformed into an aesthetic piece of work, an art form. The final result is usually an evocative story or poetry. Some authors name this form of writing creative nonfiction. Creative nonfiction is a term coined in the 1960s and 70s from journalism. Creative nonfiction tells a story grounded in data, defying the tendency towards rigid reporting of facts. It demands the employment of "techniques of fiction for its compelling qualities and emotional vibrancy". Writing creartive non-fiction This demands that the writers "inject

[1] https://theautoethnographer.com

themselves" by including more of their own experiences while imaginatively delivering the data faithfully in such a way as to deepen the reader's understanding of the topic (Caulley, 2008; Cheney, 2001). The word creative in creative nonfiction draws flack as it may imply that something is made up or is pretentious. The opposite is the case for creative nonfiction, which is meant to portray the authenticity of the author's experience in a vivid, compelling manner so that the reader derives infinite possible associations through a hermeneutic understanding of the texts[2]. In creative nonfiction, there lies what Gutkind (2012) calls the "cardinal rule" in which the writer pledges to the reader that cannot be violated: "You cannot make this stuff up!" The term nonfiction provides the premise that the writing is about something that has happened. It is an aesthetic evaluation of data collected based on a phenomenon experienced by the writer. What is transmitted in the form of creative nonfiction is the subjective truths behind these experiences.

The phenomenological attitude towards creative nonfiction writing is driven by the wonder of perception of how situations appear, show, or present themselves to the observer. In the observer's encounter with the field, phenomenology directs its attention to the origins of meaning and understanding through the observer's felt experience or Erlebnis. Unlike a summary narrative or report where the reader is provided with content of the situation that had occurred, creative nonfiction provides the aesthetics of the atmosphere of the event, its sights, smells, sounds, taste, and sensations, putting the reader themselves in the situation. The purpose is not to provide direct answers or analyses, but rather, it is a method of questioning, assimilating, and understanding. Phenomenological research begins with wonder, explores prereflexive, prepredicative experiences, aims to grasp the identity/essence/otherness of a phenomenon, requires the observer to bracket pre-judgment of the event, studying with the attitude of the epoché (the bracketing of one's cultural prejudices), the reduction and the vocative (Van Manen, 2016, pp. 27-28).

Theorizing Stories

As the writing takes shape in the course of the research, theory begins to emerge. In the Autoethnographic and CAP methods, there is reciprocity between theory and writing. We use theory to inquire about and explain the nuances of an experience. At the same time, the story, the creative nonfiction, is a mechanism that illustrates how these nuances are sensed and felt (Adams et al., 2015, p. 90). The writing brings theory to life. The theory does not add to the story or vice versa; rather, theory and story exist symbiotically, frequently told together. Ellis (2004, p. 194-196) highlights

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[2] I would use the word creative in a manner that philosopher Salomo Friedlaender applied to the term, Creative Indifference, from which the German word for "creative" is schöpferisch, which connotes something fertile, from which possibilities can arise.

that "there is nothing more theoretical or analytic than a good story". Storytellers employ analytic techniques that allow them to interpret their world. They evoke, through the story, which is art, this world that they have experienced.

> The artwork is, (…), a thing that is made, but it says something other than the mere thing itself is, *allo agforeuei*. The work makes public something other than itself; it manifests something other; it is an allegory. In the work of art something other is brought together with the thing that is made… The work is a symbol.
>
> Heidegger (1971, p. 19)

Unlike reporting or narrating facts, evocative storytelling evokes scenes stimulating the senses. The use of metaphor in storytelling is integral to human understanding, affecting human thought and behaviour (Lakoff & Johnson, 2008). The story is a basic system in our mind that helps us organise most of our experiences, knowledge, and thinking. We use stories to make sense of our relationships. We project one story onto another to make sense of the unfolding story. "The projection of one story to another is parable" (Turner, 1996, p. v). The touching of human hearts and minds through stories is ingrained in human history. How would the message of the bible still resonate through generations if it were not for the stories written in the old and new testaments? How could we grasp the teachings of Jesus Christ had he not told them in parables? Neuroscience validates what our ancestors intuitively realize that stories have power (Armstrong, 2013). In traditional Chinese medicine, evidence of the use of storytelling for emotional healing is documented in the work of Confucian educator Wang Fengyi (1864-1937) (Fruehauf, 2009).

How does story enrich theory? In the examples featured in this paper, the author writes in two distinct voices. The storyline that describes the author being immersed in the event taking place is written by the author from their first-person point of view. Written in the present tense, the reader gets a first-hand account of the situation. The second feature of the writings is the parts where the author writes about the event from a third-person point of view. This part is the author's interpretation as they write the paper. It is the theoretical part of the writing which explains "what is going on in the story?" and "how does one make meaning of it?" This part of the writing varies in flavour, depending on the dissertation's focus.

On Writing the Lives of Others

The Autoethnographic style is always written with the "other" in focus. In Autoethnography, that "other" is the culture of the society or group in the field. In Therapeutic Autoethnography, the "other" is the client, and sometimes other individuals that may be part of the client's therapy situation (e.g., a translator or co-therapist).

> The presence of an other in autobiographical and biographical texts means that they are always written with at least a double perspective in mind: the author's and the other's. The eye of the other directs the eye of the writer.
>
> Elbaz, (1987, p. 14)

With Therapeutic Autoethnography, the intent is to learn about the client's life through the therapist-researcher's experience in the field of the therapy session. Representing a life is being able to validate that person's objective and subjective markers. This belief in introducing the client as a "real person who is present in the world" is part of a long history of sociology that develops methodology allowing for subjective verbal and written expressions of meaning given by these subjects so that we may experience aspects of the other's inner lives (Sartre, 1963). Unless used creatively and formed as aesthetics, the spoken and written word is limited in describing this intimate "seeing" of the other.

The Autobiographical Experiencing

Unlike in the English language, the notion of "to experience" in German is differentiated by two words, *die Erfahrung* and *das Erlebnis*; *Die Erfahrung*, implying between travelling (*Fahren*) and standing still, and *das Erlebnis*, between living (*Leben*) and death. The sense of *das Erlebnis* belongs to the aesthetic realm. The immediate, passive, undifferentiated, pre-reflexive nature of *das Erlebnis* is an un- and yet-to-be-integrated inner experience, which is to be differentiated from the cumulative, cognitively realized sense of *die Erfahrung* (Elsaesser, 2009). Gadamer (1960/1975) alludes to the concept that *das Erlebnis* is an embodied experience that becomes part of one's whole life, the (auto)biographical that constitutes us:

> If something is called or considered an *Erlebnis*, that means it is rounded into the unity of a significant whole: An experience is as much distinguished from other experiences—in which other things are experienced—as it is from the rest of life in which "nothing" is experienced. An experience is no longer just something that flows past quickly in the stream of conscious life; it is meant as a unity and thus attains a new mode of being one. Thus it is quite understandable that the word emerges in biographical literature and ultimately stems from its use in autobiography. What can be called an experience constitutes itself in memory.
>
> Gadamer (ibid., p. 57)

The experience, *das Erlebnis,* of being in contact with the co-created field in the therapy setting becomes life to the persons who have been touched by it. That which has touched us with *das Erlebnis* is unforgettable, indeterminate, and an irreplaceable part of our whole being.

> (E)ssential to an experience is that it cannot be exhausted in what can be said of it or grasped as its meaning. As determined through autobiographical or biographical reflection, its meaning remains fused with the whole movement of life and constantly accompanies it.
>
> Gadamer (ibid.p. 60)

Gadamer goes on to explain how that which is aesthetically experienced is *Erlebnis*. The concept of *Erlebnis* is the determining feature of the foundation of art. The embodiment of *das Erlebnis* of the phenomenological field in the psychotherapeutic situation is an aesthetic experience. Aesthetic experience is the essence of experience, a mode of understanding the self. We begin to understand ourselves through our aesthetic experience of the other's phenomenological field. "Self-understanding always occurs through understanding something other than the self and includes the unity and integrity of the other. (ibid., p. 83)." Aesthetic experiencing the phenomenal field is the foundation for intersubjectivity, wherein experiencing the other leads to a hermeneutical understanding of the self.

Facts, Facticities, Fiction and Truth in Writing

Denzin (2013) addresses the issue of autobiographical truths. There are different types of truth. Facts refer to events that have occurred that are in-disputable, e.g., a client is of a certain age when he entered the session and was diagnosed by a psychologist with depression. Facticities (Husserl, 1913/2012; Merleau-Ponty, 1962/1945) describe how the same facts are experienced by a client, e.g., how the client feels about being 30-year-old man, living in a camp with other refugees and getting a high school education in Vienna. Fiction is a creatively crafted narrative, a story, that handles real or imagined facts and facticities. Truth, then, is anything which is in agreement with facts and facticities. Reality consists of the 'objects, qualities or events to which true ideas are' directed (Peirce, 1958, p. 74). A truthful fiction (narrative) is faithful to facticities and facts. "It creates verisimilitude, or what are for the reader believable experiences." Denzin, (2013, p. 29).

In Autoethnography, the prerequisite is not fiction but nonfiction, which is true fiction. Denzin quotes Elbaz:

> Fictions […] merely arrange and rearrange events that could have happened or did happen. Realist fiction, for example, presents its narrative in a way that is made to appear factual, that is, as a linear, chronological sequence of events […] autobiography is fiction and fiction is autobiography: both are narrative arrangements of reality.
>
> Ebaz (1987), quoted in Denzin (2013, p. 29)

Research Design Overview

While the research methodology of Therapeutic Autoethnography is adapted from Autoethnography, its research design has its specifications. The "Autoethnographer" in Therapeutic Autoethnography is the researcher, practitioner and psychotherapist of the studied field. The field is defined as the therapeutic encounter, including all persons in the therapy session and situation. Even though the study is about the client's psychotherapeutic process and centred around the client's story, the field extends beyond the individual. The definition of these parameters differentiates Therapeutic Autoethnography from Autoethnography traditionally applied.

The Psychotherapist is the Researcher

In Therapeutic Autoethnography, the autoethnographer is the psychotherapist-practitioner of the therapy situation. The Autoethnographer is the research tool. Conversely, In other forms of empirical research, the instruments come in the form of questionnaires, surveys, statistics, or even theory (Richardson & St Pierre, 2018). The researcher takes seriously the epistemic claims to knowledge, as well as the aesthetic practices of imaginative, creative, and artistic craft that are integral to the research process.

Therapists learn from therapists. The experiences of practitioner therapists are an invaluable resource of authentic educational material. Professional therapists, those who are considered adequately and formally trained in accredited institutions and countries, have universally been and will continue to be, educated through direct transfer of experience from mentors, who are the more established practitioners. Throughout their careers, psychotherapists also maintain their work competence by continually learning from other psychotherapists by attending supervision and peer group supervision, ultimately picking up essential skills from mentoring therapists and colleagues. Learning through the experience of fellow professionals is a life-long process for therapists. Therapeutic Autoethnography promises to provide a critical knowledge resource for therapists and student therapists keen to up-skill. More than just static information of theory and techniques, autoethnographic writing is rich in the human experience of the therapist-researcher from whom we want to learn.

What are the Therapist Effects?

The famously documented study by Wampold & Brown (2005) reveals that the individual therapist determines the quality of the psychotherapeutic outcome. Their study included 581 practitioners and 6000 clients. They discovered how the age, gender, and diagnosis of the client had no impact on the treatment success rate. So did the therapists' experience, training, and theoretical orientation. These factors do not affect the efficacy of their therapeutic work. They found consistent disparity in

the client outcomes between the most and least effective therapist[3]. This phenomenon confirmed the concept of a much earlier observation by David F. Ricks (1974), who first coined the term, supershrinks, to describe highly skilled therapists. "(I)n the chemistry of mental health treatment, orientations, techniques, and even medications are relatively inert. The catalyst is the clinician (Miller et al., 2014).

Considering that the variable that determines good client outcome is the person of the therapist, valuable learning will be obtained from knowing *who* the therapist is *while they are in session with the client*. The emphasis here is important, as we are interested in how the therapist as an organism lives and breathes while immersed in the field and not in the superficial attributes or cultural status of the person. The therapist becomes the observer who participates in the psychotherapeutic process of the client. The more phenomenological the observation, the more attuned the therapist is to the field. The more evocative the writing, the more depth of understanding can be mined from their experiences. The data from their observation is taken from information recorded in the therapist's session notes and the therapist's introspective recollection of the sessions. The writing process is deliberate, requiring the therapist-researcher to consciously reflect on their experiences throughout the process. The therapist is the protagonist in the story, written in the first-person perspective. The scenes are written in the present tense. The written style follows the principle of show-and-not-tell.

Writing in the first-person voice and following the show-and-not-tell principle takes the reader on a journey *into* the field so that they may also experience the field. When presenting the case study, the therapist also interweaves commentary on the story. The commentaries are written from the third-person perspective. The content of the commentary can include insights, hindsight, theoretical analyses, and a critique of the story. The two perspectives from the author co-exist as a polyphony, an internal dialogue, a conversation of the past in the present, a heuristic process.

Attuning oneself to the co-created field, the researcher is the observer in the field. The observer interacts with and integrates with the field as they enter it. Through this interaction, the observer and the field begin to transform. There is a co-creation of the field. The researcher entering the field participates in that change with a self-reflexive attitude and puts this experience into creative writing. The primary rationale for autoethnography is to engage the therapist-researcher's voice in the writing process. Through writing, the researcher fleshes out their lived, embodied experience of being in the field of the session with the client. In place of "objective"

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[3] "However, clients of the best therapists in the sample improved at a rate at least 50 percent higher and dropped out at a rate at least 50 percent lower than those assigned to the worst clinicians in the sample." Scott D Miller in The Secrets of Supershrinks (https://www.scottdmiller.com/wp-content/uploads/2014/06/Supershrinks-Free-Report-1.pdf)

accounts and analyses of the situation in the therapy session, autoethnographers situate themselves in an engaged subjective position (Poulos, 2021). The therapist-researcher's felt experiences in the field are a fertile source of information, which has the potential to capture the rich nuances of the psychotherapy session from the perspective of the therapist. From this, we learn deeply from the therapist-researcher about them at work with the client. Autoethnography is the study of the person of the researcher (Adams et al., 2015, p. 66). The work becomes viable when the researcher engages within the field. In Therapeutic Autoethnography, this engagement is that of the therapist in contact with the field of the alliance with their client. The writing process requires the therapist to be intentional in their attunement to the phenomenon of the field. This premise is also, incidentally, the therapeutic contract in humanistic psychotherapy.

The Field of the Therapeutic Encounter

The field represents the subject of study into which the researcher enters. In Therapeutic Autoethnography, the field represents the therapy encounter in the therapeutic situation. The situation is not limited to the set-and-setting of the therapy. The therapist's first contact with the client often happens before the first appointment. The therapist also makes contact with the client when they consider this client's work outside the sessions and in supervision. Every element that exists in the therapy situation, where both the client and therapist meet, is the field and has relevance. Research begins with the therapist-researcher being subjected to the field of the therapy situation with that client.

The word "field" is associated with different levels of meaning, as highlighted by Staemmler (2006). We use the word 'field' in describing objects: 1) literally, like fields of corn, 2) extendedly, as in a football field, 3) figuratively, as in a professional field, 4) in the scientific arena, we think of a magnetic field in physics 5) in psychology, the field implies mutually interdependent facts or phenomena. In Autoethnography, the field represents the totality of the observed culture, encompassing the individuals and all parts of their sociological and cultural components. There are nuances to the meaning of the term, which indicate the interconnectedness of discrete parts – as in the above examples, the corn plants, the different individuals in a profession, and an electrical source—that make up a whole. Lewin (1951) describes the field as "a totality of coexisting facts which are conceived of as mutually interdependent." Gestalt psychologists Wertheimer (1963), Köhler (1929/70), and Koffka (1936/2013) introduced their idea of 'isomorphism', that "our experiences and the structures that underlie have the same structure," also describing the connection between the biology of the organism and the environment in the field.

One may be better able to grasp the idea of "field" by observing ants in ant nests as described in Hofstadter (Hofstadter, 1981). The organized nature of an ant colony may be compared to human societies, except that "this organization does not arise

from higher level decisions, but it is part of a biological cycle." Ants observed as individuals seem to be autonomous. When we focus on these creatures as a colony, we can appreciate the structure of their "field". Individual ants as part of the colony react to stimuli from the environment. In contact with a food source or danger signals, the individuals in the colony react somatically. These individuals spontaneously produce pheromones and move their bodies in response to these stimuli. Pheromones and physical contact with each other affect individual ants in the field, which in turn respond and affect the other ants. The ant nest is more than the sum of the individual ants because of the field.

The field is also experienced as "biological knowledge" (Goldstein, 1939/1995) and "intra-organismic" (Perls et al., 1951/1984). This could imply that as humans, we make decisions consciously and are also manoeuvred by biological knowledge outside our consciousness. Within the field lie mutually interdependent processes that connect the organism and the physical environment. Since it involves mutual interdependence of individuals, we can also see the social aspect of colonies. Parallel to human communities, we may appreciate how a person can be described as being "of their own field". Lewin (Lewin, 1951) used the term "life space (L)" to describe the combination of the individual organism (P) with their psychological environment (E) as it exists for this individual.

$$L=P+E$$

Consider that for each individual organism, there is a continuous flow of data dynamically being received and presented at the same time from the organism and to the field. Human communities form what Hofstadter (1981) terms "representational systems", which are "active, self-updating collection of structures organized to 'mirror' the world as it evolves", like countries, organizations, cultures, families. These systems appear to define themselves through psychological decision making of individuals. Psychological decisions made are in response to environmental and social interactions. The environment and societies return feedback to the organism. The environment and societal influences affect the individual (P) physically. Individuals act in a way motivated by own needs in response to their psychological environment (E). The presence of this individual in the field is their life space (L). This argues the notion that the individual can never be a discrete part of the field but is of the field.

Therapeutic Autoethnography is a method that studies the field of the therapeutic situation. The life space (L) of the therapist in the session with the client is the person of the therapist (P) integrated with the client plus the environment of the therapeutic situation (E). The field of the therapeutic encounter is co-created.

Change that happens to the client happens with a change in the field. Whatever changes the field transforms the therapist. Therapeutic Autoethnography harnesses

the experience (*Erlebnis*) of being subjected to the therapist's life space of the field. The experience is aesthetic. The change experienced by the therapist is indicative of the change in the field, including the client. The writing is the art form that captures this change. The art form is *eine Erlebniskunst*, based on (the researcher's) *Erlebnis*.

The Research Subject

In conventional empirical research in psychotherapy, the client, and often only the client's pathological symptoms, is regarded as the subject of study. In Therapeutic Autoethnography, the client, integrated with the co-created field of their psychotherapeutic process with the therapist, is regarded as the research subject.

They are, however, not the only subject of study due to the co-creation of the field with their therapist. Nevertheless, depending on the intended recipient of the research thesis and the tone of the research question, the focus of inquiry may seem to weigh on the client. One would expect that since the premise of using Therapeutic Autoethnography is for psychotherapy research, the interest would be on the client's process that is perceived through the phenomenological experience of the therapist. Since the research method integrates the therapist's lived experience, *das Erlebnis*, the autobiographical aspect exists.

The Data Source

In Therapeutic Autoethnography, the field notes are the therapist's session notes. These are the journals and notes that therapists in practice routinely record in text form. Other materials include intake forms, recordings, text messages, artwork and documents that relate to the client being in the session. During the writing process, the therapist's introspective recollection of the session events makes up a significant part of the data.

Methodological Integrity Considerations

The emphasis in writing out the methodology of Therapeutic Autoethnography is to give shape to its foundational principles. "Story is about principles, not rules." (McKee, 1997, p. 8) Principles are how things can work, regardless of time and context. Principles are different from rules, which imply compliance, insisting on doing things in a certain way. The difference is crucial. The work of Therapeutic Autoethnography need not be modelled after any well-written work by others but rather be formed within the principles that shape the intention for doing the work of therapy and research. True artists do not strive to break the rules— that is the work of unschooled, rebellious activism. True artists master their craft through form. Autoethnography is research based on art. Forming a sense of aesthetics might help us to create good work with this method.

In evaluating the methodological integrity of qualitative studies in psychotherapy research, (Levitt et al., 2021) write:

> Methodological integrity is established when research procedures and designs act to support the goals of specific research projects (i.e., the research questions or aims); respect the approach to inquiry in use (i.e., epistemological framework); and are tailored for the characteristics of the subject matter (e.g., the complexity of the subject and the ability of data sources/participants to participate in research and be insightful) and the investigators (e.g., their identities, lived experiences, resources). Methodological integrity can be evaluated by the consideration of fidelity and utility, as will be described.
>
> Levitt et al. (2021, p. 71*)*

These philosophical principles bring possibilities for methods that involve inquiring with aesthetics, as we can define the parameters within which the method exists. While using Therapeutic Autoethnography, the key is to define the subject matter that has utility for the profession and to maintain fidelity to the subject.

Research Worthiness in Therapeutic Autoethnography

The quality criteria for this method depend on the credibility of the written case studies, the authentic reflection of the situation, and the transferability of the concepts. Creative and analytical writing of case studies does not constitute research per se. The position put forth by researchers for qualitative methods of the CAP ethnographies can be applied to Therapeutic Autoethnography.

Namely, there is the requirement for the work produced to

1. provide a substantive contribution to the understanding of the field of psychotherapy process (encompassing client and therapist dynamics, methodologies, and psychopathology),
2. demonstrate grounded scientific perspective of the writer,
3. display aesthetic merit, that opens up the text, inviting interpretive responses, and is artistically shaped, satisfying, complex and not boring,
4. exhibit reflexivity, showing clearly the author's subjectivity, self-awareness, and self-exposure so that the reader can make their judgements of their point of view,
5. impact both researcher and readers, generating an emotional and intellectual response, generating more questions and inspiring action.

Quality Criteria in Story Writing

For the aim of the research, the case study stories require:

1. people depicted as living characters are developed from persons in their authenticity,
2. a description of the aesthetics of the context, setting, and environment,
3. an epiphany or crisis that provides dramatic tension, a plot, a series of events that revolve around and toward some form of resolution,
4. a temporal order of events that is true to the actual case,
5. a point or theoretical learning or meaning that arise from the experience of reading the story (adapted from Denzin, 2013).

The Show-and-Not-Tell Principle

The show-and-not-tell principle helps the writer to stay true to the phenomenon of the events recalled, by sifting out language that communicate analyses and pre-judgment of the writer. This is a practice in phenomenology. To show-and-not-tell involves confining to descriptions of our sensory perceptions, i.e., the scents and odors, sights, tastes, physical / tactile feelings, and sounds. Explicit mention of emotions, and feelings (of emotions), and intellectual analyses of the protagonist are carefully minimized and best left out. This may sound contradictory to the work in therapy and research, but it is crucial to the storytelling. Here is an example of the difference between using show-and-not tell principle (example 1) versus describing the feeling (example 2) and describing thoughts (example 3).

Example 1: following the "show-and-not-tell" principle "May I come in?" He whispers standing at the slightly opened doorway of the therapy room. I show him in. I'm oddly conscious of being aware of my body movement, as if I were encountering a being that would be startled at a wrong move.
Example 2: describing emotions "May I come in?" He whispers standing at the slightly opened doorway of the therapy room. I show him in. I'm feel a tinge of nervousness, perhaps fear of startling him.
Example 3: describing thoughts (interpreting) "May I come in?" He whispers standing at the slightly opened doorway of the therapy room. I show him in. I move my body very slightly, for fear of seeming confrontive to him.

The examples above show how subtle the differences seem to be, between using the principle of show-and-not-tell and describing feelings and interpreting. The differences, however, affect reader's experience of the story, and the reader's relationship with the protagonist in the story. By leaving out descriptions of emotions and interpretations, the writer gives the reader space to think and feel the story. The reader who is allowed their own sensations and interpretations of the scene, enters the scene, and engages with the field.

Sometimes, space is given when we omit writing about feelings or thoughts, and just leave the void, as in the differences between example 4, 5 and 6. The blank space in these examples describes more accurately the flavor of the impact the client's words has on the therapist.

Example 4: leaving an empty space (marked in **) "My head aches all the time." Ranee rolls her eyes. "I'm always on the screen. My eyes are burning. I'm working, searching all the time…I Google *everything*." "Everything," I say. * *
Example 5: describing emotions (marked in **) "My head aches all the time." Ranee rolls her eyes. "I'm always on the screen. My eyes are burning. I'm working, searching all the time…I Google *everything*." "Everything," I say, *feeling bewildered*.
Example 6: describing emotions (marked in **) "My head aches all the time." Ranee rolls her eyes. "I'm always on the screen. My eyes are burning. I'm working, searching all the time…I Google *everything*." "Everything," I say. *I cannot how exhausting it must be for her*.

Maintaining the show-and-not-tell principle, is an attempt at staying in a phenomenological attitude. One may need to take a little more time to notice the nuances of the physical environment, the atmosphere, and sometimes apply the use of metaphor.

In the case studies presented below, the principle of show-and-not-tell is mostly adopted. There are also instances where descriptions of emotions and interpretations are necessary. Example 7 demonstrates the expression of emotion "to be tickled".

Describing the physical sensation "tickled" helps to bring the phenomenon to life. One can literally feel it. This is a subtle contrast to Example 8 and 9, where we are "telling" the emotion "I feel".

Example 7: "My mind starts to think of all probabilities and permutations, and I cannot sleep," she says, eyes wide open, agitated and gesturing with her head. "Probabilities and permutations," I say. Repeating the words, I cannot help but be tickled by what I just said.
Example 8: "My mind starts to think of all probabilities and permutations, and I cannot sleep," she says, eyes wide open, agitated and gesturing with her head. "Probabilities and permutations," I say. Repeating the words, I cannot help but find this funny.
Example 9: "My mind starts to think of all probabilities and permutations, and I cannot sleep," she says, eyes wide open, agitated and gesturing with her head. "Probabilities and permutations," I say. Repeating the words, I cannot help but feel like laughing.

Principles are not rules. They are ideals put in place to better support the premise of the work.

Ethical Considerations

Ethical challenges over procedural, process, and relational ethics, highlight the importance for the therapist-researcher, primarily to address each case uniquely, involving continual monitoring and circumspection. We cannot be mindful enough of the risk which belongs not only to the client, who may feel shamed, exposed, or judged from the literature, but also to the professional life of the therapist, who may be judged for having their work out there for all to evaluate.

In the case studies, identifiers of the characters in the stories have been masked. Part of the strategy is to create composites of different individuals. Masking identity is a general practice, and it also does not address ethical issues entirely (Tolich, 2010).

Protection of Human Research Subjects (2009) stipulates:

> (A)n investigator shall seek such consent only under circumstances that provide the prospective subject or the representative sufficient opportunity to consider whether or not to participate and that minimize the possibility of coercion or undue influence.
>
> Tolich (2010)

Likewise, informed consent and retrospective informed consent is not sufficient a measure for protecting the subjects under study from potential conflict of interest since there'll always exist, in relationships, power differentials.

The general rules of engagement proposed for this method, should align with the scholars like Denzin (2013), Ellis (1995), Medford (2006) and Tollich (2010):

1. to choose the topic carefully before embarking on the writing,
2. to treat all subjects, including the researcher as vulnerable,
3. to publish nothing that one would not wish for the persons being written about to read, even though the characters are masked.
4. to adopt the phenomenological attitude of looking at what is, with positive regard and compassion, showing and not telling, affords an extra layer of assurance of ethical soundness.

If unconditional positive regard for all in the stories is not possible, it is best to refrain from publishing. Publish what holds truth and relevance, for as long as a period as possible, for the target audience. What is published is inked like a tattoo. Publish only for the enrichment of the field. As we learn to do this, we remember that our primary obligation is to psychotherapy as a profession, the therapists, and the clients we study.

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Results: Two Case Studies

"Every person's life is worth a novel."

Erving Polster

Results

The following sections are case studies to illustrate how Therapeutic Autoethnography is applied in the study of the psychotherapeutic encounter. As the author, I am the practitioner and researcher in both cases. The case studies are written as stories in the form of creative non-fiction. The data presented in these studies are based on the therapist's notes, routinely recorded in a journal minutes after each therapy session's end. At the time of the sessions, using these cases as material for an inquiry was not planned. The cases from the numerous others in the handwritten records were selected based on the investigated theoretical theme.

Case study #1 is entitled "Ghost", which is a story of the therapeutic process of a client named Randall [4]. Randall was previously hospitalized due to having experienced a psychotic incident. He suffers symptoms of generalized anxiety and shows signs of having paranoid ideation. This case study explores the therapist's process with a client that left the therapist with unfinished business after several years after its ending. This case study serves as an example of how the therapist can use writing as a means of self-supervision while inquiring into the phenomenon of the therapeutic field and psychopathology.

Case Study #2 is entitled "I Google Everything", which is a story recounting the therapy session with a client named Ranee, who was diagnosed with obsessive compulsive disorder (OCD) and irritable bowel syndrome (IBS). This case study demonstrates the use of Therapeutic Autoethnography to provide insights into the therapeutic process in an aesthetic form. These studies also demonstrate how such an artistic form of information transfer can provide a deeper understanding of the therapeutic work and the phenomenon of OCD than other empirical methods like reporting, narrating, or quantitative research.

To facilitate a flow between story and theory, I place myself, as an author, in two meta-positions: in the first person recounting the event and in the third person commenting on the story, in a polyphonic weaving of voices. The first-person voice tells the therapist's story in session with the client. The third-person voice speaks from the viewpoint of the writer. Both voices are meta-positions; each embodies the same person, the therapist-practitioner and the researcher-autoethnographer. Both describe experiences of the same scenes but at different time-and-space. Narrating the story, I address myself in the first person, telling the story in the present tense. Writing in the present tense and the first-person perspective brings the events for the reader to the fore, inviting them to live the experience. Through this writing process,

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[4] The actors in this case study are given pseudonyms, and identifiers to their persons have been masked. The client is not aware of the writing of this article, as this case closed years before the writing of this paper.

more insights emerge. Writing the case studies, besides the heuristic experience of acquiring new recollections and insights of the sessions through telling the story, pieces of theory come to mind. The theory is written in the third person perspective using the past tense. This theoretical analysis is interwoven with the story. The theory reinforces the theme of the story.

The product of this endeavour is the case study stories written below. The outcome of this project is the demonstration of the method. It also demonstrates how extensive knowledge can be mined from the therapist's notes, recollection of the events, and reflexive writing.

***

Case Study #1: "Ghost"

Ghosts are spectral, illusive, sensitive, and ephemeral beings. We live amongst ghosts. Ghosts haunt us. They seem to yearn to be felt and acknowledged. Oftentimes we experience being ghosts ourselves whenever we feel lost, detached, and unreal. What is it like to live the life of a ghost? I am reopening this case which took place more than five years ago. Since then, I have moved back to Singapore after having lived for 15 years in Austria. Not long after, the COVID-19 pandemic happened. This case remained for me an "open gestalt" destined to be written in this thesis. It has an ambiguous and somewhat discomforting ending— the kind that many therapists might agree to dread. Some may even go as far as to render this case a failure. Revisiting this piece of work is nevertheless irresistible to me.

Aims and Scope

Writing this case study as creative non-fiction, using the method of Therapeutic Autoethnography, is an exercise aimed at discovering the complexity of the contact-making process in therapeutic work. The client is documented to have experienced a psychotic episode two years before that required a two-month lengthy hospitalization. He suffers from generalized anxiety. He displays paranoid personality traits, which were perceptible from his manner of conversing with the therapist during the session. There is an awareness during the therapeutic encounter of the client's dis-ease at being in dialogical contact. Making contact is a daunting experience, even for the most confident of us, as we are naturally vulnerable at the contact boundary.

In the therapeutic relationship, however, the therapist and client meeting at this contact boundary is the agent for therapeutic change. Healing through dialogue is an eminently hermeneutical phenomenon (Gadamer in Staemmler (2009, p. 65)). Writers acknowledge the significant moment of change in therapy as a consequence of contact as the "transcendental phenomenon" (Clarkson, 2002), "miracle moments" (Santos, 2003), "sacred moments" (Pargament, 2007), the "moments of meeting" (BCPSG, 2010), and the I-thou moment (Buber, 1970/1936). The work with Randall is about supporting him towards the contacting process.

This case study describes the client's therapeutic process through the phenomenological lens of the therapist, whose modality of therapeutic work is Gestalt therapy. Writing this story, the therapist takes on a new role of researcher, making it a deeply, intentionally introspective process. The story is written as a show-and-not-tell, a technique which serves to transmit the sense of aesthetics of the field sans unnecessary interpretations from the researcher, leaving the reader open to taking in their own associations of the events that unfold in the story. The premise of this method of inquiry is to derive or enhance theory from the story, or what we can also term theorizing story. The reflections on the story and the writing process are written in the third person. The readers of the story may have different takes on the

reflections, as these insights are the by-product of a hermeneutic-type internal dialogue.

The Subjects, Diagnosis and Setting

The Subjects in this case study story include 1) the client, Randall, who was at the time completing his master's degree at a university in Vienna. He was on the final leg of his term there and had planned to return home to Paris for good, where he lives with his parents. Randall had been hospitalized for two months in a psychiatric ward in his hometown two years prior, having suffered a psychotic break. He was diagnosed with "acute polymorphic psychotic disorder without symptoms of schizophrenia", the ICD-10 code F23 (World Health Organization, 1992). Randall sought treatment at the psychotherapy clinic for acute anxiety symptoms, which threatened to cause him another breakdown. He was worried about not being able to complete his studies due to the symptoms. 2) The therapist, who is in her late 40s. She is a Gestalt therapist who worked with the clinic. The therapy sessions were held at a psychotherapy clinic where the therapist worked. The client was assigned to the therapist after he completed an intake interview with an intern and an intake therapist. The client and therapist often work in one of many rooms at the clinic. The room configuration, therefore, differs in different sessions.

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"Once upon a time there was a little prince
who lived on a planet scarcely larger than himself,
and who was in need of a friend."
For those who understand life,
it would have sounded much more true.
You see, I don't want my book to be read lightly.
It pains me so much to relay these memories.
[…].
If I'm trying to describe him here, it's in order not to forget him."
The Little Prince (de Saint-Exupéry, 1942/2015, p. 16)

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For the full paper please contact the author.

Case Study#2: “I Google Everything”

This case study, "I Google Everything", is an aesthetically written form of creative non-fiction about the therapeutic encounter with a client named Ranee. The methodology used for this inquiry is Therapeutic Autoethnography. This method of inquiry introduces an approach to a "way of seeing", of understanding, and the "way of sharing", the process of uncovering the suffering encased in the client's symptoms of OCD and IBS. Ranee's therapeutic process is a story worth telling. It sheds light on how relevant the initial contact of the first session with the client is to the therapeutic process. The essence of "suffering" is the experience that is usually overlooked and frequently sidetracked with intake procedures. The work provides insights into the phenomenology of OCD and how, in this client's case, the symptoms are associated with transgenerational-type trauma. This paper discusses how trauma is treated experientially using a version of Gestalt therapy chair work.

The value of treating psychopathology as a process is highlighted in the conclusion part, demonstrating how this differentiates effective psychotherapy practice from the medical profession that traditionally relied on diagnostics to design treatment.
This case study demonstrates how Therapeutic Autoethnography can be used in research to bring complex theoretical concepts of psychotherapy to life, particularly psychopathology—providing a powerful enhancement to our understanding of psychological suffering beyond the prevailing mono-personal attitude of clinical diagnosis.

Aims and Scope

This study is an aesthetic inquiry, through Therapeutic Autoethnography, into the therapeutic process of a client diagnosed with OCD and IBS. This study aims to gain new depths of understanding of the phenomenology of the client’s experiencing OCD symptoms and the trauma that is underlying. The writing provides insights into the client’s therapeutic process, revealing perceptible therapeutic change and the meaning of change in this client’s process. The study is empirical in so far as to provide insight and a felt sense of the phenomena that is grounded in data. It does not aim to satisfy postpositivists' requirements that assume the possibility of obtaining objective, quantifiable results or that such results are of use for this nature of the inquiry.

The Subjects, Diagnosis and Setting

The Subjects in this case study story include: 1) the client, Ranee then a 25-year-old technology researcher who arrived in Vienna from India weeks before we met. The client was diagnosed by a psychiatrist in India a year before with OCD and IBS. She did not take the medications prescribed to her. When she arrived in Vienna, she

suffered adjustment issues leading to anxiety attacks. Her symptoms heightened further as she continued being in Vienna. She experienced insomnia, nausea, pain in her abdomen. She had compulsively cleaned lavatories, even public ones to find relief. 2) The therapist, who's in her late 40s. Born in Singapore, she had lived and worked in Vienna for more than a decade. She is a Gestalt therapist in private practice, and the narrator of this case study, and author of this paper.

The therapy sessions were held at the private practice in Vienna.

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For the full paper please contact the author.

Published Case Studies

"This article will be of value to therapists seeking to capture their experience of staying with a client's suffering when this suffering cannot be put into words."

Reviewer

Introduction

At the time of writing this dissertation, I have published three papers in 2022 presenting case studies using the Therapeutic Autoethnography method. Here are the excerpts of the just the stories written in the articles. I am deeply grateful to the editors and peer-reviewers of these journal for their acceptance of the work, and the support they have rendered to me during the review process. I have learnt much from their professional feedback.

The three papers are:

1. *Writing evocative case studies: applying autoethnography as a research methodology for the psychotherapist*, published by The British Gestalt Journal (Chew-Helbig 2022a). This paper features the story entitled, "Scream".
2. *A psychotherapist's lived experience in-session with an asylum seeker and translator: An autoethnographic case study,* published by European Journal of Psychotherapy & Counselling (Chew-Helbig 2022b). This paper features the story entitled, "Undercurrent".
3. *Understanding Salomo Friedlaender's Creative Indifference: A Psychotherapy Case-Study,* published by Geštalt Zbornik (Chew-Helbig 2022c). This paper features the story entitled, "Stuck".

These papers demonstrate the potential for using Therapeutic Autoethnography as a method for enquiry into psychopathology and illuminating theoretical aspects in psychotherapy.

"Scream"

"Scream" is a story written in the British Gestalt Journal in 2022, as an introduction to the method of adapting Autoethnography in Psychotherapy research. It is entitled, *Writing evocative case studies: applying autoethnography as a research methodology for the psychotherapist* (Chew-Helbig, 2022a). This article demonstrates how the story is presented and how it is used to illuminate aspects of Gestalt therapy theory.

Demonstrating the power of aesthetics in the therapy process as well, the story tells the tale from an online therapy session with a client diagnosed with borderline personality disorder. Writing the story, I discovered the how during the first session, I had recorded in my notes and had overlooked, the client's first words to me, "I'm in a bad place." I was also able to appreciate how this was figural for the client. He was in that place and there was something about being there. I was also drawn, by phenomenon, and subjected to this figure, though I could not grasp its full meaning. It was only many sessions later, during an online session, when we were oceans apart that this "bad place" made sense to us. The phenomenology of the borderline personality which is true for this client is captured in this story.

An Excerpt from "Scream"

'I am in a bad place,' Matt said at the start of our first session together. He had come dressed all-black in t-shirt, cropped pants, and a baseball cap; the kind of gear one would wear as a crew member on a film set. His voice has a somewhat 'sing-song' character to it, which I associate with spoken Cantonese. This was five months before this session in the car.

'Tell me something about that place,' I remember saying, feeling as if I were being led towards a threshold, beyond which is an intangible space. Now, I am not sure if he had heard what I had said then, for we promptly got ourselves lost somewhere in the conversation of that intake session. We didn't directly address that 'bad place' that day, but it lingered amongst us like fertile ground. This is where I find myself standing at every session, at this threshold, with Matt always on the other side. I begin to feel the pain of that bad place. Sometimes we'd sit together in silence, oftentimes he'd reach out. My heart wrenches and I feel the urgency to pull him out, over to my side. Each time I reach out, I find no grasp.

Having to take our sessions online is as much of an impediment as it is a relief. The distance buffers me from sensing the pain of being at that threshold in its intensity. At the same time, I feel painfully inept.

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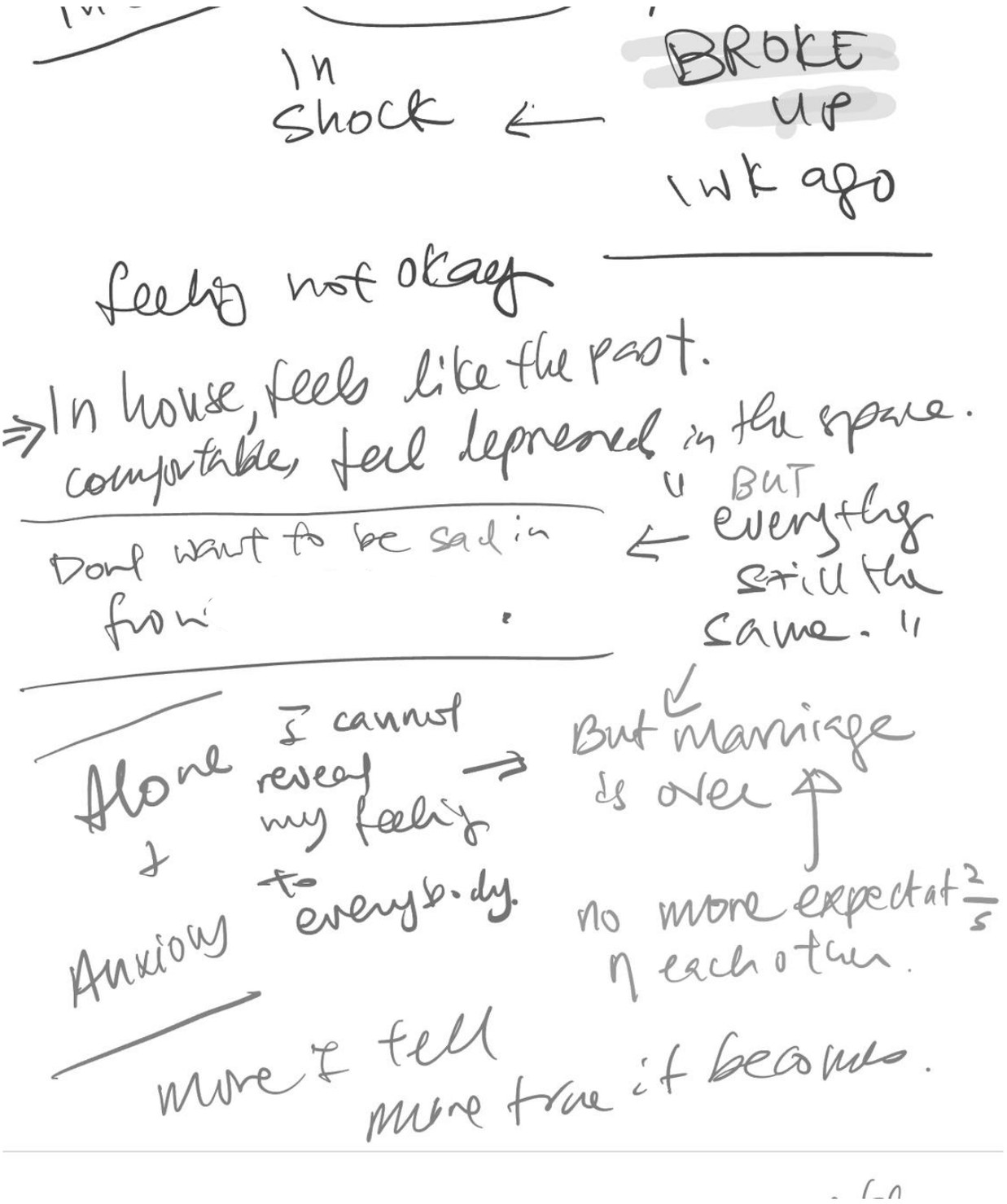


Figure 19 Excerpt from therapist's notes. "Scream - broke up."

***

‘Do you mind if I light up this cigarette?’ He asks.

‘No, of course not,’ I say. The irony of that question.

He turns the ignition on in order to wind down the window nearest to him. The skin on his cheeks twitches on contacting the icy midnight air. He lights up, and inhales deeply into the cigarette.

‘I had time last weekend to work on my latest film,’ he says. It is going to be an Arthouse type of film, somewhat different from his usual commercial documentary film projects. ‘The entire film is going to be about a butterfly girl trapped in a cocoon. Her birth, long overdue. The mother gods have forgotten about her. She finds herself alone, screaming in the dark, but no one hears her.’

‘Beautiful,’ I say, deeply impacted. ‘Beautiful.’

‘Yes, and the funny thing is,’ he says, ‘you will not hear a sound of her screams in the film.’

In that moment, I hear a scream. It wasn’t a sound, but rather a deep reverberation in my chest. I see Matt look back at me from the PC monitor. He has wound the window back up. He is once again in an enclosed car. I find myself standing back at that threshold. This time, I see it. I see that place. That is the place where, for years, the boy endured the blows from his elder brother while shielding his younger sister from them, the place where he was made to feel sorry for, and then give in to the sexual needs of a schoolteacher. The boy knew that he was left there alone. He cries. He looks at the clock on the wall. He cries on. He wonders when someone would come for him.

A ping from Matt’s phone distracted us. We suddenly become aware of the time. It’s now almost the end of our session.

‘Matt,’ I ask him, ‘I am wondering if you, too, feel like screaming?’

‘All the time,’ he says.

‘What stops you from screaming right now?’ I ask.

‘H…huh… I can’t do that,’ he says.

‘Too bad that we’re meeting remotely this time,’ I say. ‘Had I been in that car with you right now, we may have had a chance to scream, together.’

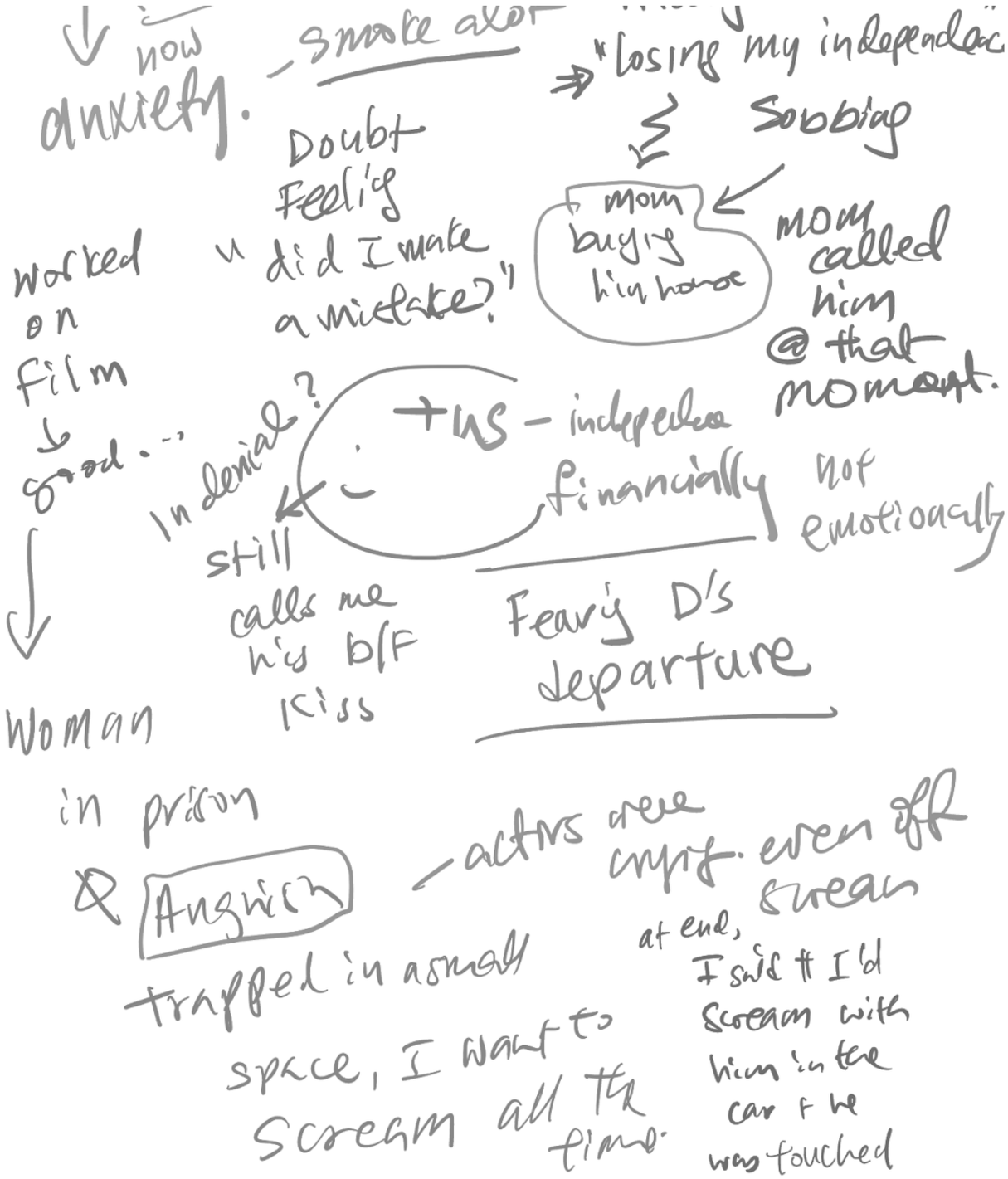


Figure 20 Excerpt from therapist's notes. "Scream"

Matt's eyes well up. 'Thank you,' he says, now sobbing. 'Thank you for saying that.'

We stayed online for a moment till it felt right to sign out.

***

A positive reviewer feedback on this article:

> I've enjoyed reading this article, feeling inspired by the theoretical feast and a compelling story. The author has a sound knowledge of the research method and the ability to inspire Gestalt audiences to pursue autoethnographic writing. Furthermore, the article has been written in a beautiful language.
>
> Reviewer (2022)

***

"Undercurrent"

"Undercurrent" is a story incorporated in an article published in the European Journal of Psychotherapy and Counseling for a Special Issue on diversity and inclusion in psychotherapy (Chew-Helbig, 2022b) entitled, *A Psychotherapist's Lived Experience In-Session with an Asylum Seeker and Translator: An Autoethnographic Case Study.*

This article features a story of my work with an asylum seeker from Afghanistan, with the help of a translator. The client is diagnosed with PTSD and was recommended therapy through social services. The case study is written with the intention for the reader to grasp the nuances and the atmosphere of an otherwise linguistically challenging therapeutic situation. What is interesting in this case is that the therapist and client do not share the same language, and they made do with experimentation and exploration of the field and including the translator as an active member in the process. The writing process also unearthed for me a phenomenon that I did not realize had happened throughout therapy. That the client had articulated the phenomenon of PTSD by repeatedly saying that he cannot move forward because of thoughts of the past.

> Ali's plot toggles between contents of his past and his future. From the former we hear of his suicide attempts, the violent treatment he endured from his uncle, the friends he had lost in the war. We hear of his concern for the women he had left behind in Afghanistan who had protected him— his uncle's wife, and the mother of his friend, also named Ali. We hear also of betrayal. Ali's future promises hope, despair, and uncertainty. What we had together was in the here-and-now, like a pivot, holding space in an undifferentiated situation.
>
> Chew-Helbig, (2022b)

The story and Gestalt therapy theory are weaved together, which makes palpable, intangible aspects of the therapeutic process.

***

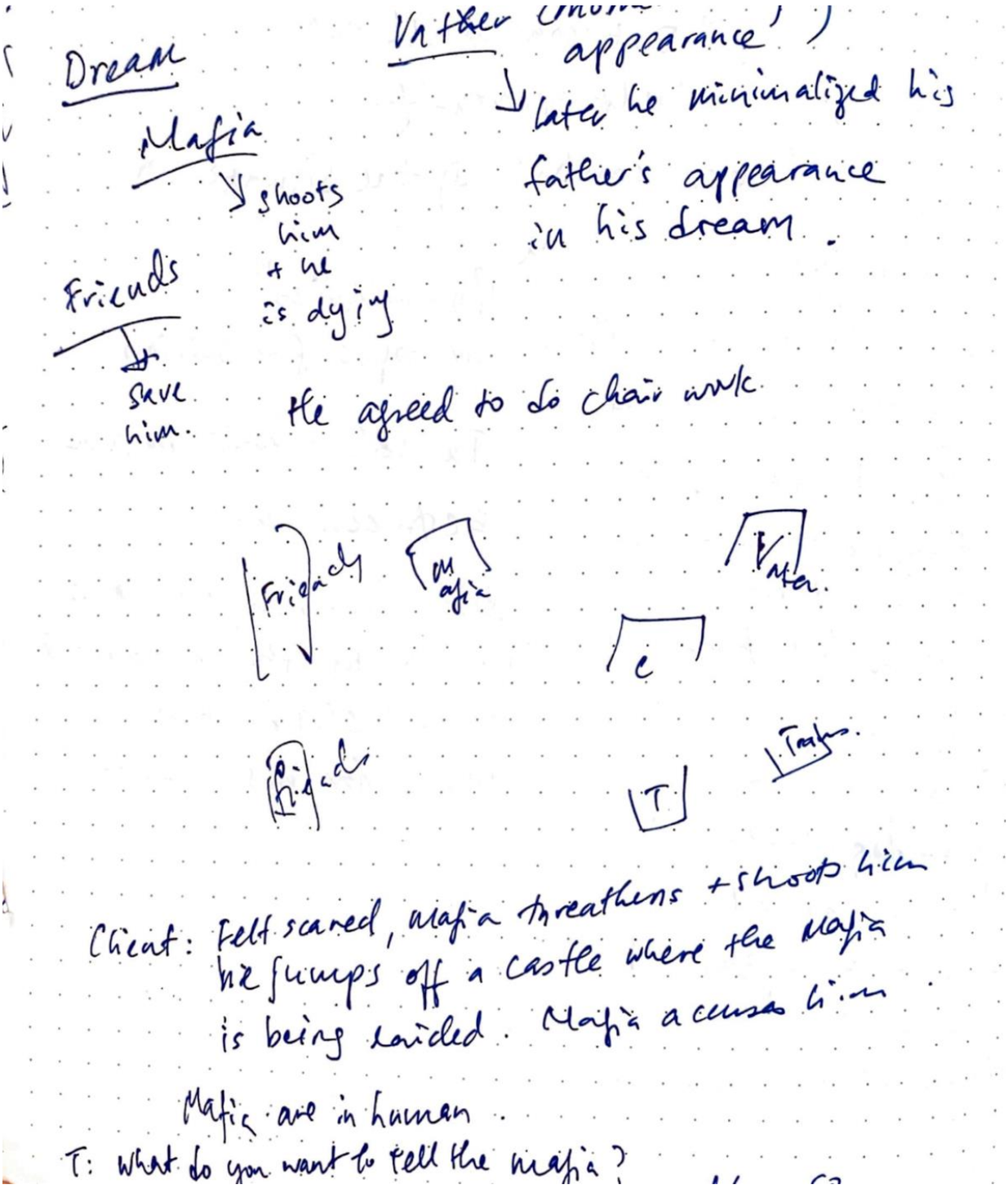


Figure 21 Excerpt from therapist's notes "Undercurrent Chair work"

An Excerpt from "Undercurrent"

"I want to be confident, but something in my past is stopping me," Zaya translates.

I give Ali a nod to acknowledge what I am hearing. He smiles back at me. His jacket hangs on a clothes rack by the door. This is the start of our second session.

Ali's opening sentence this morning feels like an invitation. Ali stands by a threshold between the future and the past. He points us in the direction of the past. I see him shudder. I shudder, though I do not know what it is I am seeing. I am interested, in earnest. I am also wary. I lean back.

***

I ask Ali what he meant by the past stopping him from being confident.

"Uncle." Ali explains that uncle is the younger brother of his father. Ali was put in the care of uncle and his wife when he was 9 years old. Till he turned 18, Ali lived with the couple and their daughter, who's a little younger than Ali. "I always hear uncle say, 'you will come to nothing. Nothing'."

"What would you say to him right now, if he is sitting here?" I ask.

Zaya translates.

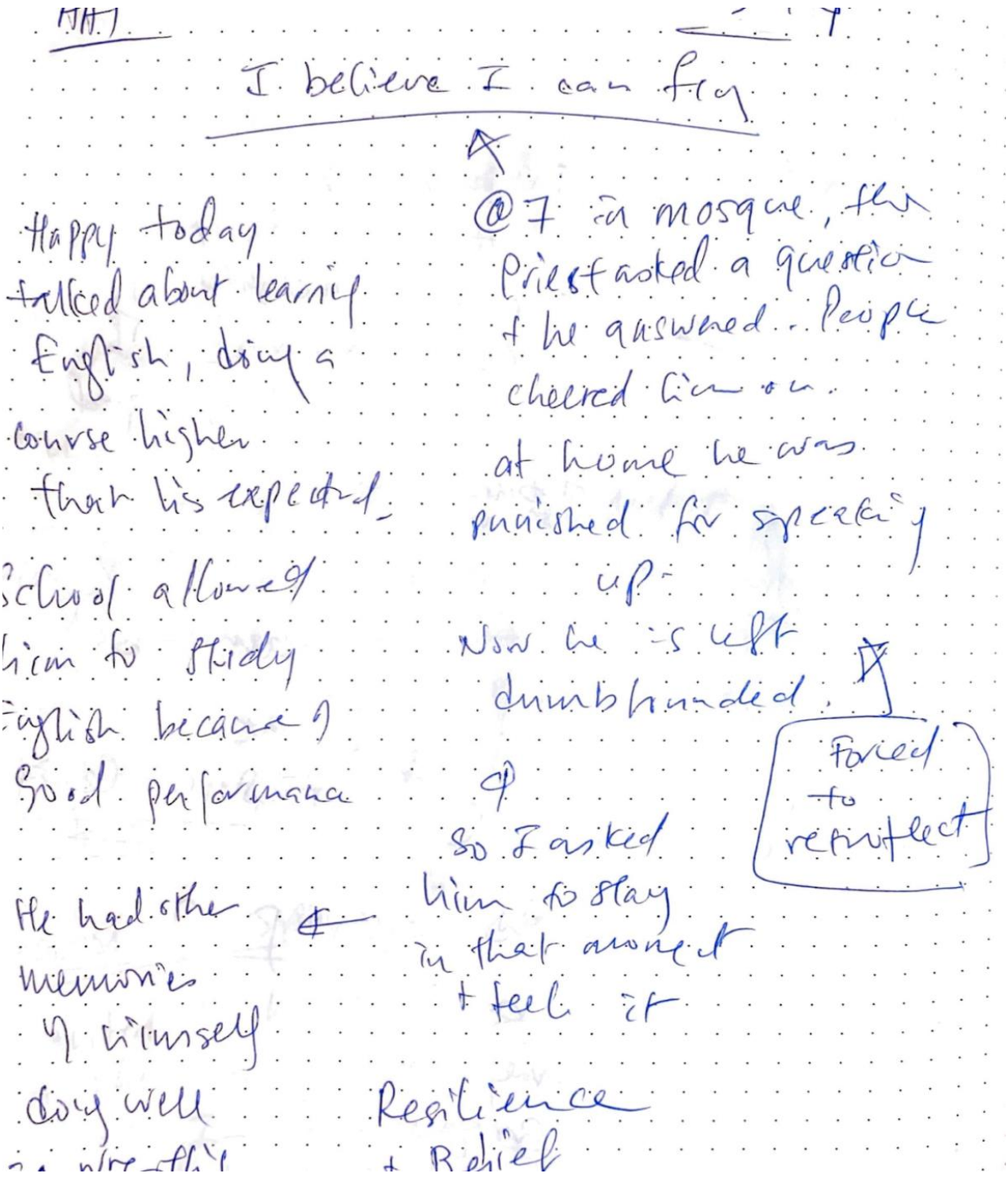
Ali clenches a fist, then promptly holds it down with the other hand. "I have no words, nothing."

"Yes." I say. I give space to the young man's rage. Zaya reaches gingerly for a filled paper cup and takes a sip. Ali and I follow suit.

I ask him how he got to be put in the care of uncle.

"After school I play football. The playground is near my house. One day, I was playing football, then the bomb of the Taliban exploded on our house." Zaya translates this, ice cold. They were all home when it happened; Ali's father Zabi, his mother, Fatemeh and his baby sister Khatere. Ali's voice cracks as he mentions the name of his baby sister. He stares, glassy-eyed, blank.

The skin on the back of my neck tingles. I glance towards the radiator by the window, as if to check if it was turned on properly. No, the air in the room is not cold. I am being touched by phantoms who have now descended amongst us. I search my bag for paper, only to find a dog-eared stack of neon coloured post-its. I write, "Zabi", "Fatemeh", "Khatere", on three little sheets, careful to get the spelling right. I lay the



I believe I can fly

Happy today
talked about learning
English, doing a
course higher
than his expected.
School allowed
him to study
English because of
good performance

He had other
memories
of himself
doing well
in wrestling

@7 in mosque, the
Priest asked a question
& he answered. People
cheered him on.
at home he was
punished for speaking
up.
Now he is left
dumbfounded.

Forced
to
reminiscent

So I asked
him to stay
in that moment
& feel it

Resilience
& Belief

Figure 22 Excerpt from therapist's notes. "Undercurrent - I believe I can fly."

post-its on the coffee table. I feel tears well up in my eyes. The space between us is saturated.

***

"How… are… you?" Ali asks me in English, then giggles.

"Thank you, Ali," I say. "I slept well, so this morning you have my fullest attention."

Zaya translates.

"How about you, Zaya," I ask.

I catch a slight blush in her cheeks, as she giggles, "very well."

"Ali says that he doesn't sleep well. For example last night he had a bad dream. A nightmare." Zaya translates.

"There is the mafia," Ali says, "who is raiding the castle. Me and my friends are hiding in it. The mafia shoots me. I'm almost dying and my friends save me. My father is there. Oh. But that is only for a short time. I fall out of the window and run away."

We share a round of chuckling. I offer Ali and Zaya a suggestion to do a chair work. Zaya's eyes lights up. I'm aware that she had read much about the Gestalt therapy method of working with dreams. "I'm not sure if you'd like to try this exercise." I say to Ali, who is probably sensing Zaya's interest.

"Yes. Yes," is his reply.

"Well, if at any time either of you are feeling unwell doing this dreamwork, say stop." I tell them.

"Yes. Yes," he says.

Zaya translates, "What do I do?"

So we set the stage with empty chairs around us. I am thankful for having booked this larger room again this morning. Ali puts a chair for "the mafia" a distant right from his. He then says that the rows of chairs stacked up along the wall of the room are where his friends sit. He takes the last chair and places it close behind him slightly to his left; there Ali seats his father.

***

Positive reviewer feedback on this article: Below is the feedback I received on the abovementioned article by a reviewer during the peer-review process. I am grateful for the generosity of their words.

> The autoethnographic approach adopted by this article is seen as a way of integrating a number of strands and concerns that tend to be left out of the more traditional case study research. By entangling theory and the story while paying attention to the direct experience of the therapist, the article attempts to capture nuances of interaction that are by definition non-verbal and non-communicative. This is a deftly written text that explores a therapeutic encounter that starts from the premise that the client and the therapist do not share a common language. While in this case, the reasons for this are specific to the biographies of the actors, what is being explored here is the possibility of something happening that cannot be put into words, which nevertheless is crucial for the therapy. By drawing on Gestalt, field theory and the relational approaches to case studies this text presents a tightly argued claim for paying attention to presence rather than representation. The success of this article is in showing that autoethnography can be a practical methodology, especially in situations where the client's suffering is caused not only by external conditions but also by their experience of being outside of language. This article will be of value to therapists seeking to capture their experience of staying with a client's suffering when this suffering cannot be put into words. It will also be a timely contribution to the emergent field of autoethnographic case studies.
>
> Reviewer (2022)

***

"Stuck"

"Stuck" is a story incorporated in an article published in Gestalt Zbornik (Chew-Helbig, 2022c), entitled, *Understanding Salomo Friedlaender's Creative Indifference: A Psychotherapy Case-Study.*

This article features a story of my sessions with a client who experiences atypical Anorexia Nervosa. This story is used to illustrate the philosophical concept of Salomo Friedlaender's Creative Indifference, or *Schöpferische Indifferenz*, first published in 1918, which is a founding philosophy of Gestalt therapy that has invaluable implications in the process of psychopathology in the psychotherapeutic process.

> Friedlaender's Creative Indifference makes explicit the "exemption of the 'inside' from utter isolation, of all dividuality", bringing the process of psychotherapy to an elevated level, where the pathos is made graspable within the phenomenal field of the therapeutic encounter. Where there is inclusion, psychopathology in the field is part of the therapist's subjective experience. Being actively indifferent, staying at the uneasy fertile void, the center of polarities and differences, in the here-and-now, allows for something to emerge. Intentional attunement to the phenomena allows suffering, which is absent, forgotten, dissociated, rejected, depressed, deflected to emerge and to be made palpable. Min's story tells of how suffering belongs not only to the client in isolation but is integral to the therapeutic alliance. This sheds light on how Friedlaender's concept of Creative Indifference provides for an invaluable resource, a philosophical framework that therapists can orientate themselves in.
>
> Chew-Helbig, (2022c)

An excerpt from "Stuck"

"I feel stuck," Min says. She has been talking about her friends, and our conversations have not gone anywhere. "I'm still not getting anywhere." Min looks at me, her bleached eyebrows in a frown, shoulders slumped.

I am struck by the contrast between her deflated posture, and her head of big dark brown curls. The look is endearing to me, like a rag doll with afro hair. It is now autumn, four months into our work together, and the air feels today, very still.

Stuck.

I know better now than to ask anything that would be answered with a shrug and an "I don't know" and be silent. I'm stuck too.

"Yes," I then say.

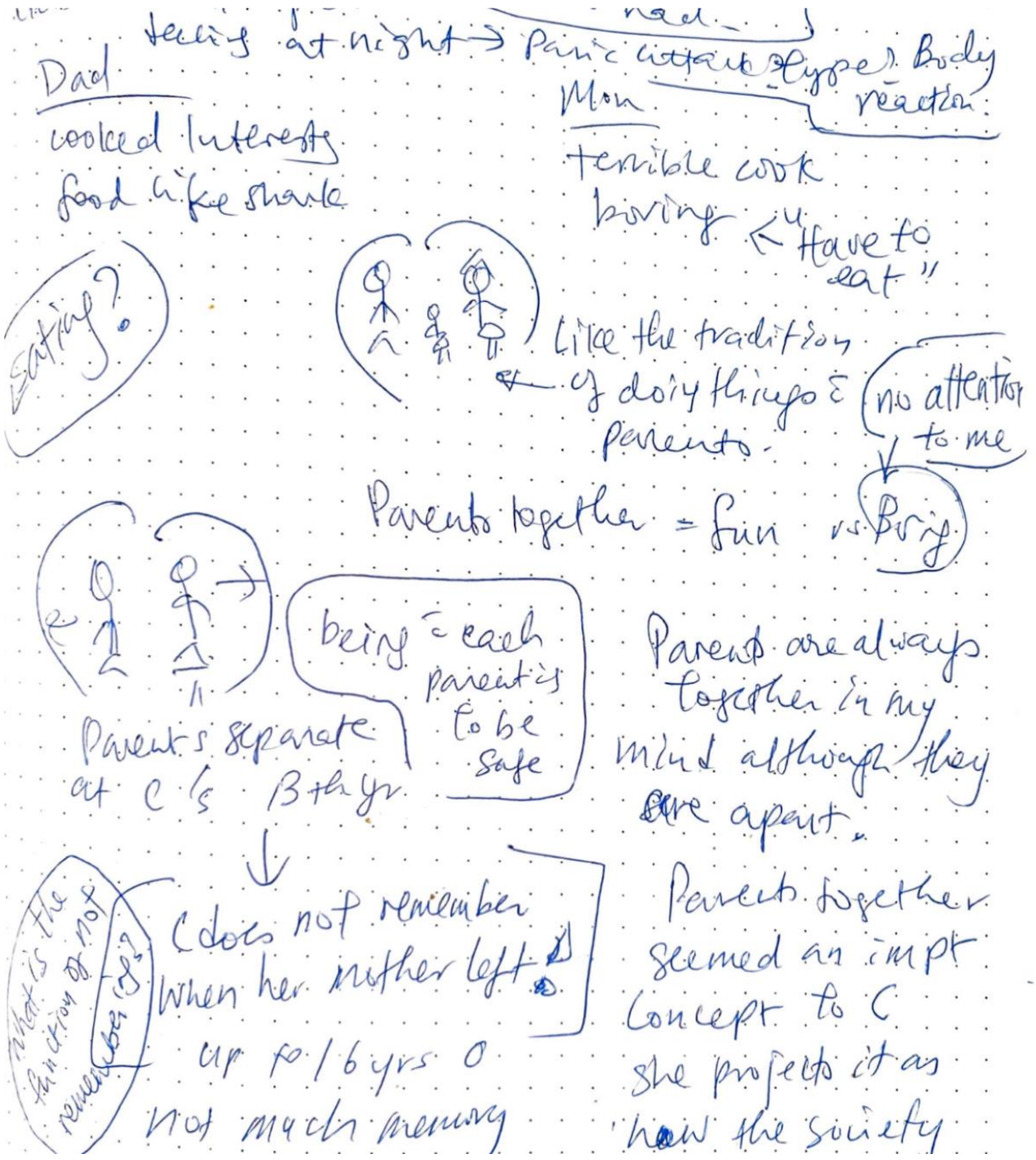


Figure 23 Excerpt from therapist's notes. "Stuck".

“It’s been a while since I came here. And I have seen many therapists,” she reveals.

I nod. In my silence, my doubts feel validated. I brace myself for something.

“I decided to see you because,” she says, her eyes turning moist for the first time since we met. She tilts her head down, perhaps in efforts to conceal her tears, “because you are the one who resembles my mother the most.”

“Oh, Min,” I say, feeling deeply touched.

The room falls silent.

I lean forward, bringing myself an inch closer to the deflated ragdoll. “Min, Min,” I say in a whisper.

She looks up.

“And I am not your mother,” I say.

She abruptly straightens up, as if startled at my words, “Oh!” She gasps.

As if we had suddenly remembered to breathe, we inhale deep breaths in unison.

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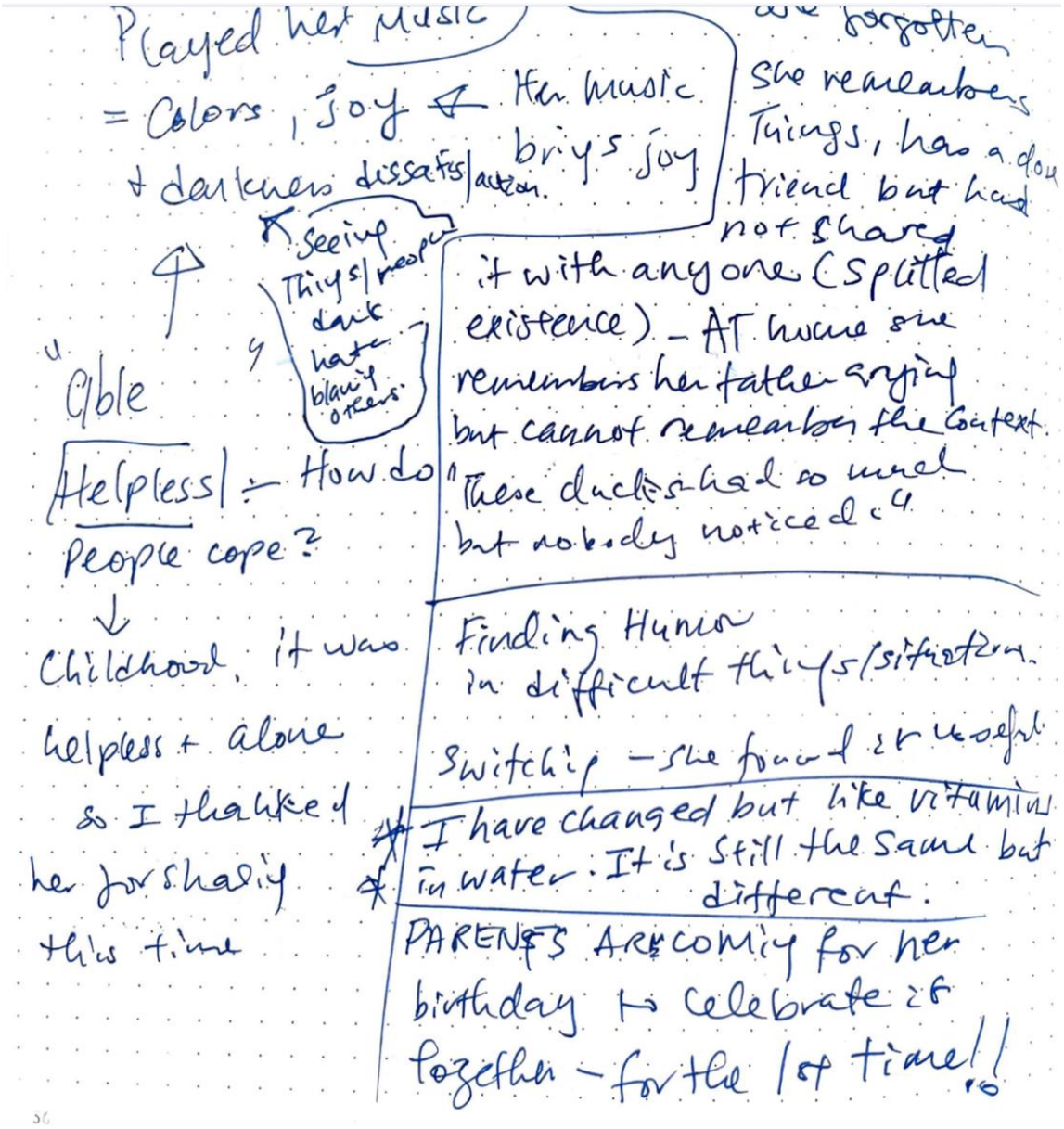

Figure 24 Excerpt from therapist's notes. Final session of "Stuck".

Conclusion

"This is a beautiful method."

China Workshop Participant (2023)

Conclusion

The clinical practice is a natural laboratory for psychotherapy research (Borckardt et al., 2008). Therapeutic Autoethnography is a research approach that fulfils the ever-present practice-research gap by allowing us to experience the therapeutic encounter in its naturalistic setting. The method engages the therapist as the researcher attuned to the phenomena of the field of the therapeutic encounter. The aesthetics of the field holds a vast amount of information about the therapy situation. This information is crucial to our understanding of the processes of psychotherapy and psychopathology. However, it eludes us. Aesthetic information is a lived experience which is neither quantifiable nor transmittable through descriptive texts alone. At most, it is relatable through art.

In Therapeutic Autoethnography, the therapist-researcher relates to the reader the aesthetics of the atmosphere of the therapy session by writing the case story. The written story is personal and reflexive, meant to evoke a sense of being in the field in the reader. The writing process is a hermeneutic one. The researcher draws the information from their memory and session notes, forming the research's grounding data. As the story unfolds for its writer, new insights and memories are ignited, developing the writing further, and establishing a heuristic process. *Heuristic* is derived from the Greek word *heuriskein*, which means "to find" or "to discover". This genre of writing opens pathways to discovery.

As it is in the therapeutic situation, the therapist is the indispensable factor because they are instruments of attunement to the field. Seeing through the practitioner's lens in the session provides a resource for inquiring deep into the therapeutic field. Traditional methods of transcript reading and video recordings of the sessions, or even expert interviewing, provide data that lack the sense of the lived experience. Therapeutic Autoethnography, on the other hand, requires the therapist to reveal the unsaid, the filtered out, the forgotten, the disowned, and the rejected; the invisible data that may shine a light on what we may understand as *therapist effects. Therapist effects* is the elusive yet crucial variable in psychotherapy research (Baldwin & Imel, 2013; Barkham et al., 2017).

When studying therapist effects, one is interested not only in *what* therapists do but also in *how* they decide to do what they have done.

Some of this information can be obtained by interviewing the therapist. However, the most valuable insights are only accessible through the heuristic process. The aesthetics conveyed in the written story bring the reader to the session's depths, offering the reader a fuller sense of what it is like to be present with the client in the field of that therapeutic encounter. Writing in this way requires skill in writing, the courage to be exposed, and the humility to allow oneself to be evaluated. The final

work should evoke emotions and be easy to read. Texts which are simple to read are often the most difficult to write.

The word aesthetics stems from the Greek word *aisthētikos*, which means 'perceptible things' and from *aisthēta*, which means 'to perceive'. Aesthetic representation and evaluation is a pre-reflexive and preverbal process of sensing the atmosphere of a therapeutic situation. The feelings or affect that arise from being in contact with the atmosphere are neither internal nor private but rather co-created. In the therapeutic setting, the atmosphere in the field is co-created between the therapist and client (and other individuals that may be present in the room). “Atmospheres are inter-subjective and holistic feelings poured into the lived environment” (Giffero, 2010/2014, p. 6).

From the case studies presented, we can grasp the atmosphere through the creative writing of the event's accounts. These accounts include more than the words spoken or the deeds done. They include thoughts, hesitations, doubts, prejudices and emotions that are not heard but felt. There is also the felt sense of the environment of the field, the taste, smell, sights and sounds. Attuning to these perceptions and describing them add to the understanding of the phenomena.

To be impacted by a client's voice, manner of speech, accent, presence and even handwriting is to be touched by the aesthetics of that *something*. This *something* that belongs in the aesthetic realm is indeterminate. To relate this lived experience necessitates a creatively aesthetic form of delivery that serves to evoke the senses.

As psychotherapists, we can experience stepping into each therapy session as if entering an unknown place. It is natural to imagine the therapeutic situation as a distinct field integrated into the broader field of our existence. Each therapeutic encounter is an embodied experience, an *Erlebnis*. Each therapeutic encounter permanently changes the client as well as the therapist. Hence the autobiographical nature of the work. This change is notable, though often overlooked. The writing process that the researcher goes through doing Therapeutic Autoethnography is an act of inquiring into this change process. This change is seen and felt from the therapist's position, although traditionally, in psychotherapy research, the focus is on the client's change process. When change happens, it happens not to any person in isolation but to everyone. Change is the phenomenon that happens in and to the field, and this field is co-created. Therapeutic Autoethnography facilitates a transfer of knowledge, which is an implicit understanding of the therapeutic situation and its change processes. Seeing change affecting the entire field is an authentic way of tracking and sensing the change in psychotherapy. Autoethnographic writing is a genre of academic writing that fleshes out change. Doing Therapeutic Autothethnography also changes the researcher. The heuristic process of writing turns the therapist into a self-supervisor. That which was erstwhile overlooked in the session, a tinge of regret, a fleeting thought suddenly comes to mind. Through these

new perspectives and insights, there is also new wisdom. The writing process is a deliberate practice that trains the practitioner to be a scholar.

As philosopher Hannah Arendt famously said, and whose words I often quote:

> What is really important to me, is to understand. For me, writing is the integral part of this process of understanding. […] writing establishes certain things. If I manage to process my thoughts adequately in writing, that satisfies me. […] I do not see myself as influential […] I see myself as somewhat of an onlooker. For if others understand the same way I've understood, that gives me a sense of satisfaction, like being among equals.
>
> Hannah Arendt in Arendt & Gaus (1964)

The defining of Therapeutic Autoethnography and the writing of this thesis has also been a journey of introspection. The distinctiveness of each therapeutic situation is not only about the client's symptoms, the therapist, or the therapeutic relationship. The therapeutic situation is embedded in its distinctive atmosphere that makes up the co-created field. The atmosphere of the field is temporal, ephemeral, indescribable and unquantifiable. These characteristics describe the qualities of change processes as well. How, then, can we fully understand psychotherapeutic change and psychopathology without considering the aesthetics of the field?

For that, we need an instrument to detect these aesthetics. In this method, the therapist-researcher is the instrument. Therapists have traditionally learned the ropes of the profession from mentors. Since the era of the shamans, the vehicle of transmission of this wisdom has always been the evocative story. Sharing stories, which has been the path into psychotherapy as a profession, will continue in this way into the future. To change this is to change the nature of psychotherapy.

Therapeutic Autoethnography is a method that has the potential for development. Its final product, the stories, make invaluable learning resources for students and other professionals. The empathic turns and wisdom from stories from the therapy room can change hearts. Therapists who may otherwise exclude themselves from research may find this method accessible. They may be encouraged to share their experiences and expertise through writing. The research world may finally come close to better grasping the essence of *therapist effects*.

Telling evocative true case stories, therapists share and learn from each other's experiences, for each therapeutic hour holds the magic of healing. Neuroscience validates what our ancestors know intuitively; that stories have power (Armstrong, 2013). Hearing and telling stories are integral in psychotherapists' work. We hear the stories told to us by the clients. We, in turn, tell them stories. Through their stories,

we feel touched by *something*. Through metaphor, we help them feel something new. Storytelling is part of the ancient art of psychological healing. An example of this is the work of Confucian educator and emotional healer Wang Fengyi (1864-1937). His work was documented in the rural Northern Chinese province of Heilongjiang. This traditional Chinese method of healing involves "talking the disease away by appealing to one's higher nature" or *xingli jiangbing*. There, the practitioner tells the people a story. As the practitioner tells his tale, the congregation would begin to weep together. The weeping turns into moans, and the moaning becomes sobs. The sobbing gets louder. Then some begin to giggle. The giggles turn into roars of laughter. There is a sense of change happening in the field (Footage of this event is documented in a video by Fruehauf, 2009).

Therapeutic Autoethnography is a qualitative method and is based on the constructivist, post-modernist paradigm. Derived from Authoethnography, it is a qualitative research method, an art-based method recognised by the APA, and increasingly used by social scientists. This thesis demonstrates the application of this method to psychotherapy case study research. Creative writing is an integral part of this research inquiry. It is grounded in naturally mined data from the therapist's session notes. While Autoethnographers are aware of potential criticism from proponents of hard science, we note that this method can also be argued as empirical in its truest sense. The word *empirical*, after all, stems from the Greek word empirikos, which means "experience". While Therapeutic Autoethnography presented in this dissertation is a standalone method, its full potential lies in its incorporation into psychotherapy outcome and process research that applies in combination with other forms of qualitative and quantitive inquiry. In traditional quantitative and outcome research design, Therapeutic Autoethnography has the potential to provide the missing link: the piece of aesthetic information that makes sense of data. For "(w)hat is hard to experience is a set of numbers. What is comparatively easy to experience is a set of qualities." Barone & Eisner, (2012) in Leavy (2017, p. 24).

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DECLARATION

I hereby declare that I have produced my thesis independently and without outside help, using only those tools which are quoted in the text and the bibliography. Every quotation I used, be it from books, magazines or the internet, has been identified in the work and included in the bibliography. Significant written or oral suggestions as well as other forms of assistance from third parties have been identified and marked as such.

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Date Signature

Abstract

Abstract (EN)

Therapeutic Autoethnography is founded on Autoethnography, an emergent field in qualitative research. Therapeutic Autoethnography engages the practitioner as a researcher in the field of the psychotherapy situation. It is a practice-based, practice-driven, practice-informed qualitative research method. It provides an in-depth understanding of what goes on in the diverse naturalistic setting of psychotherapy practice. In Therapeutic Autoethnography, the field being investigated is the therapy situation, which encompasses the individuals attending the session, the therapeutic alliance, and its environment. In the role of researcher, the therapist creates aesthetic accounts of their experiences in the field with their client(s). The writing process is deliberately reflexive and integral to the method. This act of creative writing is a heuristic process where new insights emerge for the researcher. This thesis demonstrates how Autoethnography can be applied as a practical methodology for psychotherapy case study research, especially for explicating the implicit nuances in therapy.

Keywords: Autoethnography, Therapeutic Autoethnography, psychotherapy research, psychotherapy case study

Abstract (DE)

Die Therapeutische Autoethnographie basiert auf der Autoethnographie, einem aufstrebenden Feld der qualitativen Forschung. Die Therapeutische Autoethnographie beschäftigt die Praktiker als Forscher im Feld der psychotherapeutischen Situation. Es ist eine praxisorientierte qualitative Forschungsmethode. Es bietet ein tiefes Verständnis dafür, was in der vielfältigen naturalistischen Umgebung der psychotherapeutischen Praxis vor sich geht. In der Therapeutischen Autoethnographie ist das Untersuchungsfeld die Therapiesituation, die die Sitzungsteilnehmer, die therapeutische Allianz und ihr Umfeld umfasst. In der Rolle der Forscher erstellt der Therapeut/ Therapeutin ästhetische Berichte über ihre Erfahrungen im Feld mit ihren Klienten. Der Schreibprozess ist bewusst reflexiv und integraler Bestandteil der Methode. Dieser Akt des kreativen Schreibens ist ein heuristischer Prozess, bei dem neue Erkenntnisse für die Forscher entstehen. Diese Dissertation zeigt, wie Autoethnographie als praktische Methodik für die Erforschung von Fallstudien zu Psychotherapie angewendet werden kann, insbesondere zur Erläuterung der impliziten Nuancen in der Therapie.

Keywords: Autoethnographie, Therapeutische Autoethnographie, Psychotherapieforschung, Psychotherapie-Fallstudie